KAISER PERMANENTE®

KAISER PERMANENTE
WASHINGTON REGION
PROVIDER MANUAL



JUNE 15, 2026

©2026 Kaiser Foundation Health Plan, Inc. All rights reserved.

Table of Contents

Table of Contents i
1. WELCOME TO KAISER PERMANENTE WASHINGTON 1
1.1. Introduction 1
1.2. About Kaiser Permanente (Washington Region) 1
1.3. Our Relationship with Network Providers 1
2. WORKING WITH KAISER PERMANENTE 3
2.1. Inform Us of Your Practice Changes 3
2.1.1. Alternative Medicine Providers 3
2.1.2. Mental Health Providers 3
2.1.3. Delegated Organizations 3
2.1.4. All Other Providers/Organizations 4
2.1.5. Termination 4
2.1.6. Add A Practitioner 5
2.2. Provider Responsibilities 5
2.2.1. Commitment To Quality Improvement 5
2.2.2. Communicating with Our Members 6
2.2.3. Notification of Emergency Hospital Admissions 6
2.2.4. Patient Experience Surveys 6
2.2.5. Practice Expectations - Timely and Adequate Patient Care 7
2.2.6. Professional Conduct 7
2.2.7. Provider Directory 8
2.2.8. Race, Ethnicity and Language Equity 8
2.2.9. Self-Treatment of Immediate Family Members 8
2.2.10. Use of Third-Party Apps for Billing Services 8
2.3. Medical Records and Documentation Standards 9
2.3.1. Authorship and Authentication of Dictated Reports 9
2.3.2. Clinical Performance Reviews 9
2.3.3. Confidentiality Language 9

2.3.4. Documentation Standards 10
2.3.5. Faxing Medical Records 11
2.3.6. Information Security Measures 12
2.3.7. Medical Records Standards 12
2.3.8. Password Security 12
2.3.9. Protected Health Information 13
2.3.10. Providing Documentation of Referral Encounters 14
2.3.11. Securing Confidential Information 14
2.4. New Provider Orientation 15
2.5. Credentialing 15
2.5.1. Initial Credentialing and Recredentialing 15
2.5.2. HDO Credentialing 15
2.5.3. Your Rights 16
2.5.4. Dispute Resolution Process (Nonreportable Events) 16
2.5.5. Appeals (Reportable Events) 17
2.6. Provider Communications 18
2.6.1. Provider Notices 18
2.6.2. Provider eNews 18
3. COVERAGE AND ELIGIBILITY 19
3.1. Kaiser Permanente Identification Cards 19
3.2. Kaiser Permanente Plans and Networks 19
3.2.1. Kaiser Permanente Health Plan of Washington 19
3.2.2. Kaiser Foundation Health Plan of Washington Options, Inc. 19
3.2.3. Kaiser Permanente Networks 20
4. MEMBER RIGHTS 23
4.1. Advance Directives 23
4.2. Interpretive and Translation Services 23
4.2.1. Policy 23
4.2.2. Payment for Services 24
4.2.3. Contracting for Interpreter Services 24
4.2.4. Minimum Interpreter Standards 24
4.2.5. Video Interpretation 24
4.2.6. Telephone Interpretation 24

4.2.7. Documentation 25
4.2.8. Interpretation Vendors 25
4.2.9. Translation 25
4.2.10. Translation Vendors 26
4.3. Informed Consent 26
4.4. Physical Restraints and Seclusion 27
4.5. Right to an Appeal 27
4.5.1. Expedited Appeals 27
4.5.2. Standard Appeals 28
4.5.3. Subsequent Appeal Levels 28
4.6. Right to a Second Opinion 28
4.7. Member Rights and Responsibilities 29
4.8. Member Concerns or Complaints 29
4.9. Reporting Quality of Care Concerns 30
4.10. Member Access and Corrections to Their Medical Records 30
4.10.1. Request for Access 30
4.10.2. Denial of Access 31
4.10.3. Parental Access 31
4.11. Member-Initiated Corrections to Their Medical Records 32
4.11.1. Request for Correction 32
4.11.2. Denial of Request for Correction 32
5. AUTHORIZATIONS AND CLINICAL REVIEW 33
5.1. Prior Authorization 33
5.1.1. Accidental or Work-Related Injuries 33
5.1.2. Durable Medical Equipment 33
5.1.2.1. Billing and Payment 33
5.1.2.2. Authorization Process 34
5.1.2.3. DME Service Delivery Standards 34
5.1.3. Prior Authorization Requirement and Management Guidelines 35
5.1.3.1. New Requests 35
5.1.3.2. Standard Visits 36
5.1.3.3. Exceptions to Standard Visits 36

5.1.3.4. Extensions 38
5.1.3.5. Authorizations for Medicare Coordination of Benefits 38
5.1.3.6. Medicare Coordination with a Maintenance of Benefits Plan 38
5.1.4. Radiology Services 39
5.1.4.1. Advanced Imaging Services 39
5.1.4.2. Imaging Services Requiring Prior Authorization 39
5.1.4.3. Imaging Services Not Requiring Prior Authorization 40
5.1.5. Referring Kaiser Permanente Members for Specialty Care and Services 40
5.1.5.1. Specialty Care Referrals 40
5.1.5.2. Emergency Room 40
5.1.5.3. Inpatient Care 41
5.1.5.4. Referring to Non-Contracted Providers 41
5.1.6. Requesting Preauthorization for Coverage 42
5.1.7. Retroactive Authorizations, Extenuating Circumstances, and Provider Referral Reconsideration Requests 42
5.1.7.1. Retroactive Authorizations 42
5.1.7.2. Extenuating Circumstances 43
5.1.7.3. Requirements for Retroactive Preauthorization Consideration 43
5.1.7.4. Reconsiderations of a Denial 44
5.1.8. Transplant Services 44
5.1.8.1. Requesting a Transplant 44
5.1.8.2. Member Benefits 44
5.2. Prior Authorization -Mental Health 44
5.2.1. Applied Behavioral Analysis 44
5.2.1.1. Coverage and Referral 45
5.2.1.2. Authorization Process 45
5.2.1.3. Preauthorization Process 45
5.2.1.4. Initial ABA Authorizations 46
5.2.1.5. Continued ABA treatment 46
5.2.1.6. Termination of Services 47

5.2.1.7. Addendums to Current Authorizations 47
5.2.1.8. Appeals 47
5.2.1.9. Contact Information 47
5.2.2. Coordinating Mental Health Inpatient or Residential Care 48
5.2.3. Excluded Mental Health Services and Addiction and Recovery Service 49
5.2.3.1. Mental Health and Wellness Excluded Services 49
5.2.3.2. Addiction and Recovery Excluded Services 50
5.2.4. Inpatient Addiction and Recovery Treatment 51
5.2.5. Inpatient Mental Health Treatment 52
5.2.6. Medically Necessary Mental Health Services 52
5.3. Clinical Review Criteria 53
5.4. Prior Authorization and Utilization Review 54
5.4.1. Routine Prior Authorization Reviews 54
5.4.1.1. First-Level Reviews 54
5.4.1.2. Second-Level Reviews 54
5.4.1.3. Notice Of Noncoverage 54
5.4.2. Urgent Prior Authorization Reviews 54
5.4.3. Utilization Reviews 55
5.5. Member Appeals 55
5.5.1. Standard Appeals 55
5.5.1.1. Non-Medicare Advantage Appeals 55
5.5.1.2. Medicare Appeals 56
5.5.2. Expedited Appeals 56
5.5.3. Subsequent Appeal Levels 57
5.5.4. Services Provided During Appeal 57
6. BILLING AND CLAIMS 58
6.1. Payment Policies 58
6.2. Claims Submission and Processing 58
6.2.1. Checking Claims Status 58
6.2.1.1. Paper Claims 58
6.2.1.2. Electronic Claims 58

6.2.2. Claims Adjustments 59
6.2.3. Claims Explanation of Payment/835 Remittance Advice & Reimbursement 60
6.2.4. Caring for Kaiser Permanente Members from Other Regions 61
6.2.5. Claims Submission Requirements 61
6.2.6. CMS-1500 Claims — Coding for Services Provided 62
6.2.7. Coordination of Benefits 63
6.2.8. Copayments 63
6.2.9. Deductibles and Coinsurance 64
6.2.10. Electronic Batch Transactions 64
6.2.11. Electronic Funds Transfer and Electronic Remittance Advice 65
6.2.12. Medicare Coordination of Benefits 66
6.2.13. Members' Financial Responsibilities 67
6.2.14. Motor Vehicle Accidents 67
6.2.15. NPI (National Provider Identifier) and Taxonomy 68
6.2.16. Outpatient Prospective Payment System/Ambulatory Payment Classifications 69
6.2.17. Paper Claims 69
6.2.18. Physician Reimbursement for Medical (Non-Psychiatric), Surgical, and Anesthesia Services 70
6.2.19. Requests for Additional Claims Documentation 71
6.2.20. Submission of Claim Supporting Documents 72
6.2.21. Submitting Secondary Claims 72
6.2.22. Timely Filing of Claims 73
6.2.23. Work-Related Injuries 75
6.3. Provider Reconsiderations Conducted Within Kaiser Permanente 77
6.3.1. Reconsideration Time Frames for Kaiser Permanente Conducted Reconsiderations 77
6.3.2. Medicare Advantage Non-Contracted Provider Appeal Rights 77
6.3.3. Claims (Non-Authorization Related) 78
6.3.4. Claims (Authorization/Clinical Review Related) 79
6.3.5. Pre-Payment 80
6.4. Reconsiderations Conducted Outside Kaiser Permanente 81
6.4.1. Diagnosis Related Group (DRG) Payment and Review (Cotiviti) 81

6.4.2. Optum 82
6.4.3. WA Balance Billing Protection Act 83
6.4.4. Federal No Surprise Billing Act 84
7. PATIENT CARE 86
7.1. Clinical Guidelines 86
7.2. Mental Health 86
7.2.1. Mind Phone 86
7.2.2. Measuring Quality of Mental Health Care 87
7.3. Complementary and Alternative Medicine 88
7.4. Medical Transportation 89
7.4.1. Emergency Situations 89
7.4.2. Non-Emergency Situations 89
7.4.3. Medical Transportation Management (formerly Access2Care) 89
7.5. Care Management Programs 89
7.5.1. Key Services 90
7.5.2. Program Outcomes 90
7.5.3. How to Refer 90
7.6. Home Health and In-Home Palliative Services 90
7.6.1. Scope Of Services 91
7.6.2. Admission Criteria 91
7.6.3. Arranging for Services 92
7.6.4. Contracted Providers 92
7.7. Hospice Services 92
7.7.1. Scope of Services 92
7.7.2. Admission Criteria 93
7.7.3. Arranging for Services 93
7.7.4. Contracted Providers 93
7.7.5. Hospice Information for Medicare Part D 93
7.8. Nursing Home Care – Long Term Care (Custodial Care) 94
7.9. Skilled Nursing Facility Care 94
7.9.1. Arranging for Services 95
7.9.2. Contracted Provider Expectations 95
7.10. Patient Education and Support Services 96

7.11. Specialty Services 97
7.12. Women's Health Care Services 97
7.12.1. Coverage 97
7.12.2. Home birth 98
7.12.3. Infertility/Fertility Treatment 98
7.12.4. Pregnancy Termination 98
8. PHARMACY SERVICES 99
8.1. Drug Formulary 99
8.1.1. Drug Formulary Exception Requests 99
8.1.2. Drug Formulary Recommendations 99
8.1.3. Formulary Tools 99
8.2. Pharmacy & Therapeutics Committee 100
8.3. Pharmacy Drug Benefit Help Desk 100
8.3.1. How to Submit a Prior Authorization Request 100
8.3.2. Clinical Documentation Requirements 101
8.3.3. If a Prior Authorization Request is Denied 101
8.4. Prior Authorization Review Timeframes 101
8.4.1. Medicare 101
8.4.2. Commercial 102
8.5. Prior Authorization for Injectable Drugs 102
8.6. Self-Administered Medications 103
8.7. Home Infusion Pharmacy Services 103
8.8. Specialty Pharmacy Services 103

1. WELCOME TO KAISER PERMANENTE WASHINGTON

1.1. Introduction

It is our pleasure to welcome you as a Kaiser Foundation Health Plan of Washington and Kaiser Foundation Health Plan of Washington Options, Inc. (collectively referred to as Kaiser Permanente) contracted provider. We want this relationship to work well for you, your medical support staff, and our members. This provider manual was created to help guide you and your staff in working with Kaiser Permanente's systems, policies and procedures. If you have questions or concerns about the information contained in this provider manual, please contact our Provider Assistance Unit at 1-888-767-4670.

This provider manual applies to coverage provided under both Kaiser Foundation Health Plan of Washington and Kaiser Foundation Health Plan of Washington Options, Inc.

1.2. About Kaiser Permanente (Washington Region)

Kaiser Permanente is a not-for-profit insurance company and part of the national Kaiser Permanente family of health plans. Washington Permanente Medical Group (WPMG) is an independent medical group contracted with Kaiser Permanente to provide care for its members. In combination, Kaiser Permanente is first and foremost a clinical care delivery system that consists of Kaiser Permanente facilities and clinicians and an external, contracted provider network that extends patient care to regions or specialties not serviced by WPMG physicians. The Kaiser Permanente model of care centers on primary care; prevention; evidence-based medicine; integration and communication among specialties; effective resource stewardship and providing the highest quality of care possible.

1.3. Our Relationship with Network Providers

Because our contracted network forms an important part of the clinical care delivery system, it is imperative that we develop relationships that align with the Kaiser Permanente clinical care model. Kaiser Permanente views physician leadership and clinical connections as the best way to achieve its goal of high quality, affordable healthcare. To this end, WPMG takes an active

role in leading physician professional relationships and supports our Provider Services teams when clinical issues or concerns arise. WPMG's goal is to develop excellent partnerships through physician-to-physician conversations that center on patient care, guided by Kaiser Permanente's clinical model. This provider manual lays the foundation for alignment and integration between Kaiser Permanente and its contracted network providers.

2. WORKING WITH KAISER PERMANENTE

2.1. Inform Us of Your Practice Changes

2.1.1. Alternative Medicine Providers

Provider data for chiropractors, naturopaths, massage therapists and acupuncturists is managed by Tivity (WholeHealth Living). To update your information, log into the Practitioner Portal to access the WholeHealth Living Payer Space. Within the WholeHealth Living Applications tab, follow the link to submit your update requests directly. For detailed instructions, see the Updating Information and Attestation section of the User Manual, downloadable from the WholeHealth Living site.

2.1.2. Mental Health Providers

Use the MHS Practitioner Add Form for active contracts only. If you do not know if you are directly contracted with Kaiser Permanente, please reach out to your organization's contracting department.

We have limited needs for contracted providers in our networks. At this time, we do not respond to unsolicited contracting requests. Please review the Contracting with Kaiser Permanente Washington Region for additional information.

Unsolicited requests will be declined if there is no business need. This includes leaving a contracted Tax ID and joining or establishing a new or non-contracted Tax ID. If you submit an unsolicited request for a new contract, you will only receive a reply if the request meets an existing network need and is approved for a new contract.

2.1.3. Delegated Organizations

Please refer to the Delegated Organizations/Providers List to determine if your organization has been approved for delegated credentialing.

These types of changes require contract and system updates. A 90-day notice is required for updates because the changes affect provider directory listings, authorizations, and claims processing and payment.

2.1.4. All Other Providers/Organizations

Submit a Practice, organization and billing changes form for the following:

Tax ID name and/or Tax ID changes*
- Tax ID name changes require a signed contract amendment and a system update
- Tax ID changes require a system update
- Acquisition – An acquisition is when an entity purchases a company or multiple companies. This change requires advance notice and a copy of the bill of sale/acquisition contract signed by both parties to update the contract.
- Assignment – An assignment is when an entity transfers the rights and obligations under its current contract with Kaiser Permanente to a new owner. If the new contract owner does not accept the existing contract terms, we cannot issue an assignment. A W9 is required to update the contract.

***Note:** Leaving a currently contracted Tax ID and joining or establishing a new or non-contracted Tax ID is considered a request for a new contract, not a Tax ID change.

Billing NPI changes (changes to NPI type)
- Adding or changing NPI numbers
- Changing NPI type

If you are enrolled in Electronic Funds Transfer, you must also notify InstaMed of Billing NPI and Tax ID changes.

Email: support@instamed.com
Phone: 1-877-833-6821

2.1.5. Termination

Individual practitioner - Use the Individual practitioner termination form to terminate a practitioner from your contract with Kaiser Permanente.

Full contract – Use the Provider Contract Termination form to terminate your full contract with Kaiser Permanente. See your current contract for notification requirements.

Delegated organizations/providers – Contact your credentialing specialist to follow the roster notification process. Refer to the Delegated Organizations/Providers List to determine if your organization is a delegated organization/provider.

2.1.6. Add A Practitioner

Use the Individual practitioner add form for active contracts only. If you do not know if you are directly contracted with Kaiser Permanente, please reach out to your organization's contracting department. We have limited needs within our network for new contracts. Review the following page for more information: Contracting with Kaiser Permanente.

If credentialing is required, please allow 60-90 days for processing once a complete credentialing application is received. You may check credentialing status periodically by contacting the Credentialing department at kpwapcq@kp.org.

We currently have limited needs for contracted providers in our networks. We do not respond to unsolicited contracting requests.

2.2. Provider Responsibilities

Kaiser Permanente contracted providers are responsible for adhering to and participating in the following processes that make up Kaiser Permanente's integrated delivery system model of care.

2.2.1. Commitment To Quality Improvement

Kaiser Permanente and its contracted providers commit to:

- practice evidence-based medicine as outlined in the Clinical Guidelines posted on the Kaiser Permanente provider website
- engage in quality improvement initiatives
- cooperate regarding quality improvement initiatives, including the use of Kaiser Permanente quality reporting data, working together to improve quality metrics data and reporting tools
- collaborate with Kaiser Permanente's Patient Safety department through Unusual Occurrence reporting as referenced in the provider manual and report safety concerns by sending an email to patientsafety@kp.org.

2.2.2. Communicating with Our Members

Kaiser Permanente is ultimately accountable for all provider communications to its members. To ensure compliance with state and federal requirements, all proposed provider communications must be submitted to Kaiser Permanente for review and approval prior to mailing. Additionally, Kaiser Permanente may need to submit the materials to regulatory bodies to obtain approval prior to use.

Submit draft provider communications for review at least 60 days in advance to:

Kaiser Foundation Health Plan of Washington
Provider Relations Compliance
PO BOX 34262
Seattle, WA 98124-1262
prcompliance@kp.org

Provider Relations Compliance will notify you in writing once your request has been received and a final determination has been made.

2.2.3. Notification of Emergency Hospital Admissions

Emergency admissions may be admitted without prior authorization if the patient is medically unstable. Notify us of the patient's admission as soon as possible using one of the following four options:

- 24/7 Hospital Admit Notification Voicemail Line: 1-888-457-9516
- Fax: 206-630-3418
- Online submission: Inpatient Admission Notification Form
- Call Emergency Patient Resources & Options (EPRO) at 1-800-337-3197 (24/7 access), which can also provide immediate assistance with:
  - Access to a patient's Kaiser Permanente medical records.
  - Reviewing nationally accepted hospital admission criteria.
  - Communicating with patients and families to explain medical necessity.
  - Support the transfer of patients to other facilities for continuity of care or higher level of care.

2.2.4. Patient Experience Surveys

Kaiser Permanente and its contracted providers work together to provide Kaiser Permanente members an excellent customer service experience.

Providers are encouraged to use industry-standard patient surveys and feedback received by the Kaiser Permanente Member Services contact center, Kaiser Permanente staff, and Patient Safety Unusual Occurrences reviews as opportunities for continuous improvement of member experience.

2.2.5. Practice Expectations - Timely and Adequate Patient Care

Kaiser Permanente works with network providers to ensure that WAC, OIC, CMS, and NCQA regulations and standards are met regarding patient care. Contracted providers are responsible for providing members with medically necessary services and following all applicable regulations and standards regarding timely member access to care.

Physicians are responsible for:

- Ensuring that physician services are available 24 hours per day to advise members about how and where to obtain necessary care
- Consulting with emergency department providers about the most appropriate setting for the level of care
- Authorizing coverage for urgent care in emergency departments, when appropriate
- Confirming with members their financial responsibility for any non-covered services
- Identifying, assessing, and establishing treatment plans for members
- Ensuring that members with special health care needs receive appropriate case management by calling 1-866-656-4183 to refer them for complex case management services
- Cooperating with Kaiser Permanente periodic requests for detailed reports pertaining to member access to care
- Ensuring that notes back to referring providers are timely
- Providing interpretive services as necessary

2.2.6. Professional Conduct

All contracted and employed providers and administrative staff in the Kaiser Permanente enterprise must conduct themselves professionally in their interactions with patients, providers, support staff and administrative staff. Kaiser Permanente follows and enforces the State of Washington Medical Quality Assurance Commission policy regarding "Practitioners Exhibiting Disruptive Behavior" in its contractual relationships and in its credentialing reviews.

2.2.7. Provider Directory

CMS, federal, and state regulations require that we maintain a compliant and accurate provider directory of our networks. To meet these regulations and provide the best care options for our members, our contracted providers are expected to provide us with all demographic changes and updates as soon as possible.

2.2.8. Race, Ethnicity and Language Equity

As a matter of policy and as a part of our values, we strive to match our contracted provider network to the cultural needs and preferences of our Kaiser Permanente membership. We attempt to keep records and perform analyses of our contracted provider network pool by race, ethnicity and language so we have the information at hand when a member makes a request based on cultural need and/or preference. Contracted provider groups are encouraged to support Kaiser Permanente in our efforts to better meet member needs by allowing and supporting us in our efforts to gather data on practitioners' race, ethnicity and language(s) and to compare our provider network to the communities we serve.

2.2.9. Self-Treatment of Immediate Family Members

Kaiser Permanente prohibits a provider from prescribing controlled substances for his or her own use and from prescribing controlled substances to a member of his or her immediate family, except in an emergency or isolated settings where there is no qualified physician available within a reasonable proximity. In addition, Kaiser Permanente medical offices will not accept orders or prescriptions from providers ordering services for themselves or an immediate family member.

2.2.10. Use of Third-Party Apps for Billing Services

Contracted providers who utilize third party payment apps (e.g., Venmo, Paypal, etc.) as a billing service must: (i) comply with applicable financial privacy laws, (ii) utilize a business account within the app, and (iii) configure the account settings to not allow public access to identifying information about the patient, payor, or services provided.

2.3. Medical Records and Documentation Standards

2.3.1. Authorship and Authentication of Dictated Reports

We authorize and require providers working within our facilities to document patient health care information in the medical record as specified by medical staff bylaws and Kaiser Permanente policy.

- All entries must indicate author and be authenticated
- Only authorized individuals may enter documentation in the medical record
- An entry may not be made or signed by someone other than the author except according to Kaiser Permanente policy
- Dictated and transcribed authentication shall be completed no longer than five calendar days post transcription
- Authors of documentation are required to clearly authenticate entries using signature or electronic identification

2.3.2. Clinical Performance Reviews

Kaiser Permanente routinely measures clinical performance and shares the results of that process with providers. The Healthcare Effectiveness Data and Information Set (HEDIS®) represents a core set of performance measures developed to understand the value of the health care dollar and how to hold a health plan accountable for its performance. The data gathered by health plans across the nation is maintained by the National Committee for Quality Assurance (NCQA). Providers may be contacted annually to send selected information to Kaiser Permanente for review.

2.3.3. Confidentiality Language

When transmitting patient information by fax, the cover sheet should include a confidentiality statement, such as:

> The documents accompanying this facsimile transmission may contain confidential information belonging to the sender that is protected by Washington state and/or federal law. This information is solely for the use of the addressee named above. You may be exposed to legal liability if you disclose this information to another person. You are obligated to maintain this information in a safe and secure manner.

If you are not the intended recipient, you are hereby notified that any disclosure, copying, distribution, or other use of the contents of this faxed information is strictly prohibited. Notify the sender immediately by telephone to arrange for return of the documents to us.

2.3.4. Documentation Standards

Our documentation standards reflect the importance of complete, timely, and accurate health information. Kaiser Permanente and contracted providers are encouraged to participate in efforts to improve documentation and coding accuracy, including the use of feedback and education provided by Kaiser Permanente to improve accuracy in documentation and coding, improve coding data, and improve reporting tools.

Kaiser Permanente expects the following concerning documentation:

- Member identifiers appear on every piece of documentation
- Entries are legible to others and are recorded in black or blue ink if on paper
- Entries are dated and authenticated by the author
- Documentation is made at the time service is provided
- Documentation must support all codes submitted
- Only standard medical abbreviations should be used in documentation
- All patient encounters, including telephone, fax, and electronic message exchanges are documented
- Documentation of any advance directives is in a prominent part of a member's medical record and includes whether a member has executed an advance directive, as well as documentation of any information about advance directives that was made available to the member

Documentation must include the following content:

- Problem list, including significant illnesses and medical conditions
- Medications
- Adverse drug reactions
- Allergies
- Smoking status
- Any history of alcohol use or substance abuse
- Biographical or personal data
- Pertinent history
- Physical exams
- Documentation of clinical findings and evaluation for each visit
- Laboratory and other studies that signify review by the ordering provider
- Working diagnoses consistent with findings and test results

- Treatment plans consistent with diagnoses
- A date for return visits or a follow-up plan for each encounter
- Previous problems addressed in follow-up visits
- A current immunization record
- Preventive services and risk screening

Primary care medical records must document:

- All services provided by a practitioner who provides primary care services
- All ancillary services and diagnostic tests ordered by a practitioner
- All diagnostic and therapeutic services for which a member was referred by a practitioner, such as home health nursing reports, specialty physician reports, hospital discharge reports, or physical therapy reports

2.3.5. Faxing Medical Records

Faxes must be in accordance with confidentiality guidelines and governed by the same authorization requirements as any other release of health care information. Information disclosed should be limited to the minimum necessary to meet the need. Contact information should be included on the cover sheet. Verify that the information being sent and recipient's fax number is correct.

You may release a member's medical records via fax in the following situations:

- Between Kaiser Permanente providers, including contracted providers, as necessary to support patient diagnosis, prognosis, and treatment
- Between Kaiser Permanente and non-contracted community providers as necessary to support patient diagnosis, prognosis, and treatment
- Between Kaiser Permanente facilities, business or operations units, and business associates to support care, operations, and quality oversight (such as for HEDIS or clinical review)
- To third-party payers as required for certification of hospitalization

Requests for medical records to entities not described above can be directed to the Kaiser Permanente Release of Information Department.

Western WA: Phone: 866-656-4184
Fax: 877-848-6896
Eastern WA: Phone: 509-241-7824
Fax: 855-414-1751

2.3.6. Information Security Measures

Authorized individuals who use information that identifies members must observe appropriate safeguards, such as keeping user identification and passwords secret, using only their own passwords for access, signing off applications when appropriate, protecting medical records from unauthorized access, and accessing and disclosing patient information based on legitimate business need to know.

2.3.7. Medical Records Standards

Our medical record standards reflect the importance of confidentiality and accessibility by authorized users only.

We require you to:

- Keep a unique, individual record for each patient
- Establish an organized record-keeping system to ensure that medical records are easily retrievable for review and available for use when needed, including at each patient visit
- Store and maintain medical records in a centralized and secure location accessible only to authorized personnel and provide equivalent security for electronic medical records
- Maintain and organize documents within medical records in a specified order
- If there is a paper medical record, ensure that documents are fastened securely within the paper medical record
- Provide periodic training in confidentiality and security for patient information
- CMS requires Medicare managed program providers to retain medical records for 10 years. Kaiser Permanente follows CMS standards for medical records retention.

2.3.8. Password Security

We encourage you to change your password regularly. The system will require you to change it every 60 days. You can change your password by choosing Change Your Password from the left navigation bar.

If you suspect that someone else is using your username and password to sign on, change your password immediately and contact your delegated

administrator, Provider Services or Health Plan Consultant to report the suspected breach.

2.3.9. Protected Health Information

Contracted providers must have documented policies and procedures for safeguarding patient privacy.

Policies and procedures should address such aspects as:

- Disclosure of health information
- Elements of a valid authorization to release health information
- Access and correction/amendment to medical records
- Handling of specially protected information such as drug and alcohol abuse, mental health, sexually transmitted diseases, reproductive care, and HIV/AIDS
- Proper disposal of confidential waste for all types of media
- Confidentiality and security training
- A confidentiality and security attestation signed by all employees

When providing services in Kaiser Permanente facilities, we require that you handle protected health information in a manner consistent with the Kaiser Permanente Notice of Privacy Practices. This notice applies only to services provided at a Kaiser Permanente facility. For services provided at a non-Kaiser Permanente facility, you may not rely upon Kaiser Permanente's notice to satisfy your own privacy notice obligations under the Health Insurance Portability and Accountability (HIPAA) Act.

To protect our patients, we strive to:

- Limit the use of protected health information to the minimum necessary for the intended purpose
- Provide our employees with confidentiality and security awareness training
- Control access to information based on the principle of legitimate business need-to-know

All information identifying our patients is confidential, regardless of the medium upon which it is stored. Only authorized individuals with a legitimate business need-to-know may access health care information that identifies patients. Individuals using health information that identifies patients must observe appropriate confidentiality and security safeguards.

You may not disclose health information to third parties without prior authorization by the patient or the patient's legally recognized representative, except as provided by law.

2.3.10. Providing Documentation of Referral Encounters

Whenever a Kaiser Permanente or contracted provider (other than a member's personal physician) sees a Kaiser Permanente member, complete documentation of the encounter must be made available to the referring provider and the member's personal physician. If the documentation is not added directly to the Kaiser Permanente electronic medical record, copies of the relevant medical records must be provided within five working days of the visit.

Promptly forwarding the records ensures that the personal physician has a complete medical record on file and that the referring provider has necessary information.

2.3.11. Securing Confidential Information

Confidential health information must be stored, transported, transmitted, handled, used, and disposed of in ways that protect the information from unauthorized access, alteration, destruction, disclosure, copying, theft, or physical damage but that ensures availability for provision of care.

You must have security measures in place to protect work areas and patient-identifiable information, such as:

- System access management
- Secured computer hardware
- Personnel clearance procedures
- Password protection of computer applications
- Secure disposal of confidential waste
- Sanctions for misuse of systems and data
- Signed confidentiality agreements
- Data backup and disaster recovery procedures
- Assigned responsibility for confidentiality and security of information
- Confidentiality and security awareness training

2.4. New Provider Orientation

See our New Provider Orientation page to ensure a successful start on your journey with us.

2.5. Credentialing

2.5.1. Initial Credentialing and Recredentialing

You must complete our credentialing process and be approved by our Credentialing Committee before providing covered services to our members.

To initiate the initial credentialing process, you must fill out a Provider Information Form (mental health providers please use the Behavioral Health Provider Information Form) to join the network or to add someone to your group. Provider Services will review and forward your information to the Credentialing department if the Provider Information Form is approved or if a contract is needed. Unsolicited submissions of Provider Information Forms will not be accepted unless an addition to our network is needed.

Applications for initial credentialing and recredentialing are only accepted through the approved universal credentialing data sources. Please submit your Washington Practitioner Application form (WPA) through CAQH. You must select "Kaiser Foundation Health Plan, Washington Region" to allow us to view/pull your application. Paper applications are not accepted.
We will notify you in writing when your application is approved. The contracting process begins once credentialing approval is completed. You must be recredentialed at least every three years.

2.5.2. HDO Credentialing

HDO locations must complete the credentialing process by completing the HDO Form to join the network or to add a new location to your group. Each location requires credentialing and a unique form to complete the process. Provider Services will review and forward your information to the Credentialing department if the HDO Form is approved or if a contract is needed. Unsolicited submissions will not be accepted unless an addition to our network is needed.

We will notify you in writing when your application is approved. The contracting process begins once credentialing approval is completed. You must be recredentialed at least every three years.

2.5.3. Your Rights

You have the right to review information submitted in support of your credentialing application and to correct erroneous information. However, you may not review peer reviews, peer references, and other peer-review protected materials. You also have the right, upon request, to be informed of the status of your credentialing or recredentialing application.

2.5.4. Dispute Resolution Process (Nonreportable Events)

Dispute resolution is available to eligible practitioners who have been denied initial credentialing or recredentialing for administrative reasons, such as failure to meet credentialing criteria, privileging criteria, or both.

Following an action by our Credentialing Committee to deny the application for initial credentialing, recredentialing, or privileges, the Credentialing Director or designee will send a notice of action to the affected practitioner.

To initiate the dispute resolution process, the affected practitioner must submit a written request for review to the Credentialing Director or designee no later than 30 days following receipt of the notice of action. The practitioner may submit additional information for consideration or request a personal meeting. The practitioner may have an authorized representative attend the meeting with him or her or on their behalf.

If additional information is submitted, we will notify the practitioner of our determination within 21 days of receipt of the information. If a meeting is requested, we will respond within 14 days after the conclusion of the dispute meeting.

If the practitioner has exhausted the dispute resolution process and is not satisfied with our determination, he or she may request mediation by submitting a written request to the credentialing director or designee no later than 14 days following receipt of our determination. The request for mediation shall constitute the practitioner's promise to pay one half of an approved mediator's charges.

2.5.5. Appeals (Reportable Events)

The appeal process is for eligible practitioners who have been denied initial credentialing, recredentialing, or privileging by Kaiser Permanente's Credentialing Committee, Board of Directors, or an authorized committee of the board. It is also the appeals process for practitioners who are the subject of an official action or recommendation for a reduction, restriction, suspension, revocation, termination or denial. This denial decision is reportable to the National Practitioner Data Bank (NPDB), and the appropriate state licensing board, in accordance with applicable law and Kaiser Permanente policy.

Following an adverse action or a recommendation for adverse action, the Credentialing Committee chair or designee will provide written notice to the affected practitioner. To appeal, the practitioner must send a written request for a hearing and a statement explaining the basis for contesting the adverse action within 30 days of receiving notice.

The hearing is conducted in the presence of the hearing officer, the Hearing Committee, and the practitioner. A record of the proceedings shall be maintained which, together with documentation and exhibits submitted by the parties, shall constitute the hearing record.

Within 20 days after closure of the hearing, the Hearing Committee will make a written report and recommendation and will forward those items together with the hearing record to the chair of the Credentialing Committee and to the appealing practitioner.

The Credentialing Committee will take action within 30 days after receiving the final report of the Hearing Committee. Notice of its action will be sent to the practitioner within 10 days.

If required by law, the Credentialing Director or designee will report the final action to the appropriate state licensing board and the NPDB. Kaiser Permanente also will send a copy of the final report to the practitioner.

2.6. Provider Communications

2.6.1. Provider Notices

Kaiser Permanente informs contracted providers via an electronic notice regarding the following types of changes:

- Provider compensation, payment policies and processes
- Provider health care service delivery, policies and processes

The communications are posted on our Provider Notices page on the provider site in a print-friendly PDF format and are available for one year. This page also includes a complete list of our networks for your reference. These communications are also listed in the monthly edition of our Provider eNews posted on our provider site.

It is your responsibility to remain updated on any changes by visiting our provider site regularly for updates to our policies and procedures.

2.6.2. Provider eNews

Kaiser Permanente publishes the Provider eNews, a monthly newsletter with information regarding business and clinical updates, provider 60-day notices, and other beneficial information for your clinic staff. Newsletters are in print-friendly PDF format and are available online for one year.

It is your responsibility to remain updated on any changes by visiting our provider site regularly to review our newsletters for updates.

3. COVERAGE AND ELIGIBILITY

3.1. Kaiser Permanente Identification Cards

We issue a health plan identification card to all members. This card lists the member's name, ID number, and health plan network and/or product name. You may use our Eligibility Inquiry tool to locate a member's cost-share information or contact the Provider Assistance Unit at 1-888-767-4670 to confirm eligibility. Please see our Identifying members page on the provider site for sample cards by network.

3.2. Kaiser Permanente Plans and Networks

3.2.1. Kaiser Permanente Health Plan of Washington

Kaiser Foundation Health Plan of Washington is a nonprofit membership organization in Washington state that offers HMO (health maintenance organization) plans. With most of our HMO plans, members select a primary care physician to manage and coordinate their care. The plan pays for covered services when a member receives care from a defined network featuring approximately 16,000 providers across Washington state, including Kaiser Permanente physicians who practice at Kaiser Permanente medical offices and area hospitals. There is no out-of-network coverage except for medically necessary urgent and emergency care.

3.2.2. Kaiser Foundation Health Plan of Washington Options, Inc.

Kaiser Foundation Health Plan of Washington Options, Inc. (registered in Washington and Idaho) is a for-profit subsidiary of Kaiser Foundation Health Plan of Washington offering point-of-service (POS) and preferred provider organization (PPO) plans.

- Point-of-service plans: this type of plan pays for services members receive from a subset of providers at the in-network level and allows members to receive care from a larger network of providers for, in most cases, higher cost shares.
- Preferred provider organization plans: this type of plan has an extensive network of providers. It does not require a primary care physician to provide referrals.

3.2.3. Kaiser Permanente Networks

An overview of the health plans offered by Kaiser Permanente is listed below. In addition to access within these networks, all Kaiser Permanente members have access to medical care at Kaiser Permanente clinics in other Kaiser Permanente regions.

NETWORK NAME	HEALTH CARRIER	PROVIDER NETWORKS	PLAN TYPE
Core	Kaiser Foundation Health Plan of Washington	Broad group of Washington state providers we contract with directly, including Washington Permanente Medical Group. Out-of-network coverage: None except emergency and medically necessary urgent care.	HMO
Connect	Kaiser Foundation Health Plan of Washington	Exclusive high performing network that includes Washington Permanente Medical Group with select medical groups and providers to provide coverage in 6 core counties. Out-of-network coverage: None except emergency and medically necessary urgent care.	HMO
Medicare Advantage HMO	Kaiser Foundation Health Plan of Washington	Broad group of Washington state providers we contract with directly, including Washington Permanente Medical Group. Out-of-network coverage: None except emergency and medically necessary urgent care.	HMO
Summit PPO	Kaiser Foundation Health Plan of Washington Options, Inc.	Tiered network product. Tier 1 is an exclusive high performing network that includes Washington Permanente Medical Group with select medical groups and providers. Tier 2 are the remaining providers in the Access PPO network that include:	PPO

NETWORK NAME	HEALTH CARRIER	PROVIDER NETWORKS	PLAN TYPE
		Directly contracted PPO providers, First Choice Providers in Washington state and Kootenai and Latah counties in Idaho and Aetna Signature Administrators network providers in remaining states. **OR** Directly contracted PPO providers, First Choice Providers in Washington, Oregon, Alaska, Idaho, and Montana and First Health Network in all other states. Tier 3 is out-of-network coverage by any licensed provider in the U.S. not directly contracted with Kaiser Permanente, First Choice Health, First Health Network, or Aetna Signature Administrators.	
Access PPO Options Federal (Formerly GHO Federal or Omni PPO)	Kaiser Foundation Health Plan of Washington Options, Inc.	Extensive network of Washington state providers we contract with directly, including Washington Permanente Medical Group and over 29,000 providers. First Choice Health in Washington state and Kootenai and Latah counties in Idaho and Aetna Signature Administrators network providers in remaining states. **OR** First Choice Health in Washington, Oregon, Alaska, Idaho, and Montana and First Health Network in all other states. Out-of-network coverage for all PPO Plans: Any licensed provider in the U.S. not directly contracted with Kaiser Permanente, First Choice Health, First Health Network, or Aetna Signature Administrators.	PPO
Elect PPO	Kaiser Foundation Health Plan of Washington Options, Inc.	Extensive network of Washington state providers we contract with directly, including Washington Permanente Medical Group and most major medical groups. First Choice Health in Washington state and	PPO

NETWORK NAME	HEALTH CARRIER	PROVIDER NETWORKS	PLAN TYPE
		Kootenai and Latah counties in Idaho and Aetna Signature Administrators network providers in remaining states. **OR** First Choice Health in Washington, Oregon, Alaska, Idaho, and Montana and First Health Network in all other states. Out-of-network coverage for all PPO Plans: Any licensed provider in the U.S. not directly contracted with Kaiser Permanente, First Choice Health, First Health Network, or Aetna Signature Administrators.	
Options	Kaiser Foundation Health Plan of Washington Options, Inc.	Broad number of Washington state providers we contract with directly, including Washington Permanente Medical Group. Covered at the out-of-network benefit level: Discounted rates from First Choice Health providers in Washington state and Kootenai and Latah counties in Idaho and Aetna Signature Administrators network providers in remaining states **OR** Discounted rates from First Choice Health providers in Washington, Oregon, Alaska, Idaho, and Montana and First Health Network providers in all other states. Any licensed provider in the U.S. without any discounts.	POS

4. MEMBER RIGHTS

4.1. Advance Directives

Members aged 18 or older and of sound mind have the right to execute, revoke, or change advance directives for medical and mental health care, and primary care physicians must document the status of these directives in the member's medical record.

Providers cannot condition care or discriminate based on advance directives, and all related documentation must be placed prominently in the record. Facilities must allow member or representative participation in care decisions if the member is incapacitated, including requests to withhold or withdraw life-sustaining treatment. More information and a workbook for life care planning are available on the Kaiser Permanente member website at: Life Care Planning - my values, my choices, my care.

4.2. Interpretive and Translation Services

4.2.1. Policy

Title VI of the Civil Rights Act of 1964 requires that individuals with communication barriers, including those who speak a language other than English, and those who are deaf, deaf-blind, or hard of hearing, receive equal access to services and information. Kaiser Permanente's contracted providers must ensure appropriate interpretive services are available.

A certified interpreter must be used when:
- The patient requests one
- A language need is noted by the referring provider, or
- A communication barrier is present

Interpretive services must be provided for:
- All appointments and covered services
- Emergency services
- Complaints, grievances, and appeals processes

4.2.2. Payment for Services

Interpretive services must be provided at no cost to the patient, regardless of coverage status. Payment is the responsibility of the contracted provider or the entity delivering care.

4.2.3. Contracting for Interpreter Services

To ensure quality and safety, providers must contract with certified in-person, video, or phone interpreters.

- Phone or video interpretation may be used for spoken-language needs.
- Sign Language requires video or in-person interpretation.
- Individuals who are deaf-blind require an in-person tactile interpreter.

4.2.4. Minimum Interpreter Standards

Interpreters must:

- Be fluent in both languages, with fluency verified by the vendor
- Be trained and certified in medical interpretation
- *Not* be family members, friends, or children, due to risks of bias, incomplete interpretation, and emotional burden

4.2.5. Video Interpretation

Video interpretation provides secure, real-time visual access to an interpreter and may be used as an alternative to in-person services. Limitations include patient's difficulty viewing the screen and limited mobility affecting communication. Providers must offer an in-person interpreter if video is not sufficient.

4.2.6. Telephone Interpretation

Telephone interpretation may supplement or replace in-person services for spoken-language needs. Use speakerphone in a confidential space.

Phone interpretation is not appropriate for Sign Language users or patients with hearing impairments. For these patients, use video or in-person services. Washington State's Telecommunication Relay Service (WATRS) is

also available for patients who are deaf, deaf-blind, hard of hearing, or speech disabled.

4.2.7. Documentation

When interpretation is provided, documentation in the patient's record must include:

- Whether communication assistance was provided
- The type and quality of the interpretive service

4.2.8. Interpretation Vendors

Kaiser Permanente contracts with several interpreter vendors. Providers without existing vendor agreements may establish contracts with any of the following agencies.

Vendor	Language	Modality	Service Area	Phone Number
AMN Healthcare	ASL & Foreign	Video	Statewide	866-871-8519
ASAP Translations	ASL & Foreign	In-person	Eastern Washington	509-747-5121
Deaf-Blind Service Center	Tactile	In-person	Seattle & Greater Puget Sound	206-323-9178
Dynamic Language	ASL & Foreign	In-person	Seattle & Greater Puget Sound	206-244-6709
Hearing, Speech, and Deaf Center	ASL	In-person	Seattle & Greater Puget Sound	206-323-5770
Language Line	Foreign	Phone	Statewide	800-752-6096
PGLS	Foreign	In-person & phone	Eastern Washington	877-897-4858
Propio	Foreign	Phone	Statewide	913-381-3143

4.2.9. Translation

Translation refers to converting written materials into another language, while interpretation involves conveying spoken or signed communication through an in-person, video, or phone interpreter. Kaiser Permanente's contracted providers must ensure that all general and patient-specific

written materials are provided in a format each patient can understand. This requirement may be met by:

- Contracting with a translation vendor to produce materials in the member's preferred written language or alternative format (e.g., braille, large print, audio, accessible PDF).
- Using an interpreter to provide sight translation by reading the material aloud in the member's primary language.

4.2.10. Translation Vendors

Kaiser Permanente contracts with several translation vendors. Providers without existing vendor agreements may establish contracts with any of the following agencies.

Vendor	Translation Type	Phone number
Access Ingenuity	Alternative format – large print, braille, audio, accessible PDF	707-579-4380
Propio	Electronic or written materials – foreign language	913-381-3143
Avant Page	Electronic or written materials – foreign language	530-750-2040
Lionbridge	Electronic or written materials - foreign language	877-277-3173

4.3. Informed Consent

Kaiser Permanente recognizes the right of members or their proxies to decide whether to accept or reject proposed medical treatment and to decide among recognized treatments. Before they exercise this right, you must provide members with sufficient information to reach an informed decision. The member's signed informed consent form or a chart note reflecting the content of the informed consent discussion provides the necessary documentation.

When a member is deemed medically or legally unable to give informed consent, then the right of informed consent accrues to the person or persons legally authorized on the member's behalf.

In an emergency, health care providers are authorized to supply necessary medical treatment without the member's prior informed consent.

Our standard regarding informed consent is consistent with state law and the principles of Kaiser Permanente as set forth in the Statement of Member Rights and Responsibilities.

4.4. Physical Restraints and Seclusion

It is our policy to comply with all applicable state and federal laws, including CMS Conditions of Participation, regarding the use of physical restraints and seclusion. Our goal is to:

- Create an environment throughout our delivery system that limits the use of restraints or seclusion to clinically justified situations in the appropriate clinical licensed and accredited spaces.
- Reduce the use of restraints or seclusion through preventive or alternative methods that focus on a member's rights, dignity, and well-being.
- Every member has the right to be free from restraints of any type that are not medically necessary. Restraints are never used as a means of coercion, discipline, convenience, or retaliation by staff. Therefore:
    - Use restraints or seclusion only if needed to improve the member's well-being and after less restrictive interventions have been determined to be ineffective.
    - Continually assess, monitor, and re-evaluate the member's condition under restraints or in seclusion.
    - Use ongoing performance-improvement processes to reduce the risk associated with restraints and seclusion by identifying and implementing preventive strategies, innovative alternatives, and process improvements.

4.5. Right to an Appeal

All Kaiser Permanente members have a right to an appeal. Members who receive a denial, reduction, or limitation notice for services may initiate an appeal within the specified timeframe as described in their notice of non-coverage or explanation of benefits. All coverage determinations that result in a denial, reduction in coverage request, or a failure to provide or make payment (in part or in whole) for a benefit, contain instructions for a member appeal.

4.5.1. Expedited Appeals

If a member's life, health, or ability to regain maximum function would be jeopardized by following the standard appeal process, a provider may request an expedited appeal on the member’s behalf without an

Appointment of Representative. The provider must make the request to Kaiser Permanente before the member receives the services in question.

4.5.2. Standard Appeals

Members who receive a denial, reduction, or limitation notice for services may initiate an appeal within the specified time period as described in their notice of non-coverage or explanation of benefits. A provider may appeal on the member's behalf if an Appointment of Representative form is included.

4.5.3. Subsequent Appeal Levels

Subsequent appeal levels vary depending on the member's health plan group. If a denial letter is issued, a member may request an external review. These processes are described in detail in the appeal attachments sent with the denial or appeal letters.

4.6. Right to a Second Opinion

All members have the right to a second opinion. Coverage for the second opinion is for evaluation and consultation and does not guarantee that the services or treatments recommended by the physician providing the second opinion will be covered. Coverage is determined by the member's Evidence of Coverage.

When requested or indicated and provided by physicians within the Kaiser Permanente network, a service may be covered when approved in advance. "Approved in advance" means a Kaiser Permanente physician has requested prior authorization and the member has received written approval from Kaiser Permanente.

Coverage of second-opinion visits:

- Is determined by the member's Evidence of Coverage.
- Does not imply that the member's plan will cover the recommended services or treatments.
- Does not imply that we will approve ongoing care with physician providing the second opinion.
- The referring Kaiser Permanente physician must initiate any diagnostic or therapeutic services. We do not cover services, drugs, and devices that are prescribed or recommended as a result of the consultation unless

they are covered in the member's Evidence of Coverage and approved by the health plan in advance of receiving the service.

The Kaiser Permanente physician reviewer will conduct a clinical review for second opinion requests beyond the initial approval on a case-by-case basis.

4.7. Member Rights and Responsibilities

We take patient rights very seriously, and we know that you, as a valued contracted provider in our networks, do as well. You can read our statement of member rights and responsibilities on our Kaiser Permanente member site.

4.8. Member Concerns or Complaints

Kaiser Permanente has an established process for handling member complaints expressed either verbally or in writing. Documenting, resolving, and reporting member complaints helps us improve the services we provide.

Members are encouraged to let us know of any concerns or dissatisfaction right away and, ideally, work directly with the provider or clinic involved. This process often results in the most expeditious resolution. However, if members prefer not to address the concern directly or do not feel the concern was addressed to their satisfaction, they are welcome to contact Member Services toll-free at 1-888-901-4636, or Medicare Member Services toll-free at 1-888-901-4600, or contact us via our website at www.kp.org/wa/feedback.

Member Services specialists can resolve complaints about health plan issues including billing, claims, demographic updates, plan rates and benefits, and enrollment/disenrollment concerns.

All member complaints communicated to Member Services are documented, and it is always our goal to inform members of the resolution within 30 days. Complaint data is routinely shared at all levels of the organization to represent "the voice of the customer" and inform quality improvement initiatives.

The Kaiser Permanente Privacy Office responds to member concerns involving breaches of privacy, confidentiality, and security, whether they are reported through Member Services or directly to the Privacy Office.

4.9. Reporting Quality of Care Concerns

Member complaints are managed through Member Services and the member complaints process. We also have an unusual occurrence system, managed by our Patient Safety Office, to review quality of care concerns. Unusual occurrence reports identify system weaknesses and help us avoid future errors and ensure safe and effective care for our members.

We identify unusual occurrences through member complaints and provider or staff reports. As a member of the Kaiser Permanente network, we expect you to report unusual occurrences that involve our members and to participate in the review of such cases.

Upon receiving a report, the appropriate Kaiser Permanente medical staff will review and recommend any follow-up actions. The case review findings will be documented for purposes of system improvement and recredentialing.

4.10. Member Access and Corrections to Their Medical Records

4.10.1. Request for Access

Members have the right to review their health care information with their providers. Providers must respond to requests for member access to medical records in a manner that complies with applicable state and federal laws. The procedures for members' access and review of their medical records and associated information must be designed to ensure the integrity of the record throughout the access and review. Providers must document the access or denial of access in the member's medical record.

Upon written request, members have the right to inspect and receive a paper or electronic copy of their health care records and associated information maintained in their providers' information systems, subject to exceptions recognized by law. No later than 15 working days after receiving the request, the provider shall:

- Make the requested Protected Health Information available for examination during regular business hours and provide a copy, if requested.

- Inform the individual if the information does not exist or cannot be found.
- Deny the request in whole or in part and inform the individual.

4.10.2. Denial of Access

You must inform the member of any delay or denial of the request under an exception recognized by law. You may deny access only when the disclosure:

- Would be injurious to the member's health.
- Would allow the member to identify any individual who appropriately provided information in confidence.
- May reasonably be expected to cause danger to the life or safety of any individual.
- Involves information compiled and to be used solely for litigation, quality assurance, peer reviews, or other administrative purposes.
- Pertains to information prohibited from access by law.
- To the extent possible, denied and disclosable information must be segregated, and you may deny access to the relevant portion only.

If you deny access because disclosure would be injurious to the member or could endanger the life or safety of others, the member may select another provider to review the information in question. This provider must be licensed, certified, registered, or otherwise authorized by state law to treat the member for the same condition as the original provider who treated the member.

The member must be notified of the right to select another provider to review the records, but the member is responsible for paying the second provider.

4.10.3. Parental Access

Parents have the right to access and authorize access to their minor children's records with the following exceptions where a minor can consent to their own care as mandated by state or federal law:

- Sexual and reproductive health care, including contraception and abortion services: Minors of any age.
- STI/HIV/AIDS testing, prevention, and treatment: Age 14 years or older.
- Substance use disorder treatment: Age 13 years or older.

- Mental health treatment: Age 13 years or older.

4.11. Member-Initiated Corrections to Their Medical Records

4.11.1. Request for Correction

Providers are required to respond to a member request for correction or amendment of medical records in a manner that complies with the state laws, federal laws, or both.

A member has the right to request a correction or amendment of their health care information for as long as that health information is maintained in the medical record. The request must be in writing. The provider may either accept or deny the correction or amendment, but you must respond to the request within 10 business days of receipt.

If you accept the correction or amendment:

- Make the appropriate correction or amendment to the record.
- Inform the member that the amendment is accepted.
- Review prior disclosures from the patient's chart to determine if the information in question has been previously released. If, yes, the corrected information must be re-disclosed to the previous recipient.

4.11.2. Denial of Request for Correction

If you deny the request:

- The member has the right to file, as part of the medical record, a concise statement and explanation for the requested correction or amendment.
- The challenged entry must be marked to indicate that the member has challenged it as inaccurate or incomplete, and the challenged entry must indicate where in the record the statement of disagreement is located.
- Erasure or deletion from the record is contrary to the intent of statutes, regulations, and standards governing health care information and to the generally accepted practice within the health care community.

5. AUTHORIZATIONS AND CLINICAL REVIEW

5.1. Prior Authorization

5.1.1. Accidental or Work-Related Injuries

Submit a prior authorization request so that it will be on file in case the workers' compensation carrier denies the claim. If the claim is not accepted by the worker's compensation carrier, Kaiser Permanente can then review the original request for coverage of services.

Note: Kaiser Permanente may not cover the service if the care for the injury is provided by a non-contracted provider.

For work-related injuries, submit claims directly to the designated workers' compensation carrier identified by your patient.

5.1.2. Durable Medical Equipment

Most but not all Kaiser Permanente plans provide some coverage for durable medical equipment (DME). Plans may have variable levels of coverage, deductibles, copayments, or coinsurance.

To determine coverage, use the Eligibility Inquiry tool. To see medical necessity review criteria, review our Clinical Review Criteria. Contact the Provider Assistance Unit at 1-888-767-4670 for information about coverage and medical necessity criteria.

5.1.2.1. Billing and Payment

Kaiser Permanente does not reimburse separately for items or services that are included in, or integral to, another covered service, consistent with applicable industry standards, including those from Centers for Medicare & Medicaid Services and the National Correct Coding Initiative, unless otherwise specified by the member’s benefit plan. DME provided as part of an inpatient stay or outpatient procedure is generally considered included in that service and is not separately reimbursable. DME intended for home use may be reimbursed when it is medically necessary, not included in another

service, and billed appropriately. Providers are responsible for ensuring billing aligns with these requirements

5.1.2.2. Authorization Process

You can make DME prior authorization requests by contacting Review Services or using our Referral Request tool.

To expedite patient care, check the CPT/HCPC code in the PreAuthorization Code Check tool. The tool will inform you if an authorization is needed. If it is needed and there is documentation indicated for the review, Attach those records to your referral request in Affiliate Link.

Kaiser Permanente has an exclusive partnership with AdaptHealth for certain core DME services. Under this relationship, AdaptHealth provides a list of core services for Kaiser Permanente HMO members.

Once your authorized request is received, Review Services will:

- Verify patient eligibility and benefits.
- Review guidelines and authorize or deny requests based on established Medicare or health plan criteria.
- Review and determine cost-effectiveness of equipment purchase versus rental.

For coordination of equipment delivery and pickup, contact the vendor.

Urgent After-Hours Needs

You may obtain urgent and emergent equipment after normal business hours (Monday-Friday, 8 a.m.-5 p.m.) by contacting AdaptHealth at 844-99-KPDME (844-995-7363). This service is available for all of the Washington region.

5.1.2.3. DME Service Delivery Standards

Our contracted durable medical equipment (DME) services providers are key members of our members' health care team, coordinating services with other Kaiser Permanente providers, such as nursing homes, home health agencies and area hospitals. These providers

procure high quality, state-of-the-art DME that ensures the safety of our members.

The DME provider will report hazardous situations or medical problems immediately to the referring Kaiser Permanente physician. The DME provider will train members and their family how to operate, care for, and maintain the DME safely and effectively, and provide a copy of all applicable warranties of equipment provided. If necessary, the DME provider will also provide training for specified Kaiser Permanente staff as well.

To ensure the safety of the member, the DME provider will ensure the equipment provided will be free of defects in materials and workmanship and will perform preventive maintenance on all DME. If DME requires repair, adjustment or otherwise malfunctions, the provider will pick up the defective equipment within twenty-four (24) hours of notification and repair it, adjust it, or provide an equivalent substitute. Patient safety is our highest priority, which will guide the provider in determining the appropriate response time for DME repair. In cases where patient safety may be jeopardized, repair or substitution of equipment will be completed as soon as practicable. If, however, the repair is non-urgent, the provider will establish cost estimates in advance of repair and coordinate with Kaiser Permanente for approval to repair or for authorization to replace the DME.

5.1.3. Prior Authorization Requirement and Management Guidelines

Prior authorization requirements vary by health plan. Kaiser Permanente must authorize all inpatient hospital care, regardless of plan type. Members who have out-of-network benefits may use First Choice Health and First Health Network providers. Out-of-network provider office visits do not require prior authorization. Some treatments may require prior authorization.

5.1.3.1. New Requests

The number of visits and duration are limited by the scope of the authorization, unless the request is for services that are noted as exceptions below. See standard visits and exceptions tables.

The start date of the authorization is dependent on the request date and receipt date. Requests for authorization should be received prior to or within 14 calendar days of the requested start date. If the request

is received more than 14 days after the requested start date, it will be considered a retroactive request and may be denied.

5.1.3.2. *Standard Visits*

When referring your patient for specialty care, you can choose the appropriate scope of care. By using one of these options you can ensure your patient is getting the right level of care.

SCOPE OF CARE	CPT/HCPC	MAXIMUM VISITS/DURATION	APPROVED ACTIONS
Consult only	99202	3/12 months	Office visits only
Second opinion only	99203	1/12 months	Office visits only
Evaluate and treat; surgery if indicated	99214	3/12 months	Office visits, routine diagnostics, in-office procedures. Evaluate and treat, and requests for surgeries can come directly from the consulting specialist on a separate authorization request.
Evaluate and treat; surgery if indicated	99214	6/12 months	Office visits, routine diagnostics, in-office procedures. Evaluate and treat, and requests for surgeries can come directly from the consulting specialist on a separate authorization request.
Oncology and radiation therapy			
Evaluate and treat; surgery if indicated	99214	999/12 months	Office visits, routine diagnostics, in-office procedures. Requests for surgeries can come directly from the consulting specialist on a separate authorization request.

5.1.3.3. *Exceptions to Standard Visits*

The following table lists exception guidelines for authorizing services. Services are subject to the member's eligibility and benefit coverage. Some services are limited or not covered by the member's health plan. For more information on a specific member's benefits, contact the Provider Assistance Unit at 1-888-767-4670.

AUTHORIZATION TYPE	AUTHORIZATION LIMITS
Mental health and wellness	Mental health, including addiction and recovery authorizations are processed through that department. For details, see Section 5.2 Prior Authorization – Mental Health
Complimentary & alternative health services	**Acupuncture:** Subject to limitations based on member's coverage. Member can self-refer to contracted acupuncturist for limited number of visits. After self-referred visits are exhausted, an authorization request from the treating acupuncturist, primary care physician, or specialist is required to authorize additional visits. **Chiropractic care:** Most patients can self-refer to participating chiropractor for limited number of visits. **Massage therapy:** Preauthorization is not required for massage therapy for most members. Massage therapy must have a valid order or prescription from the member's ordering provider. The number of visits is limited by the member's coverage and benefits. **Naturopathy:** Subject to limitations based on member's coverage. Member can self-refer to contracted naturopathic practitioner for limited number of visits. After self-referred visits are exhausted, an authorization request from the treating naturopath, primary care physician, or specialist is required to authorize additional visits. For more information, see Section 7.3 Complementary and Alternative Medicine.
Diabetic education	999/12 months
Diabetic retinopathy	999/12 months
Dialysis	999/12 months
Durable medical equipment	DME, prosthetics, and orthotics are processed through Review Services. For more information, see Section 5.1.2. Durable Medical Equipment.
Nutritional counseling	6/12 months
Occupational therapy	When preauthorization is required, up to 15 visits upon initial request. Subsequent visits in 15-visit increments up to the member's benefit limit.
Oncology (including chemotherapy and radiation therapy)	999/12 months
Phenylketonuria (PKU)	15 visits/12 months
Physical therapy	When preauthorization is required, up to 15 visits upon initial request. Subsequent visits in 15-visit increments up to the member's benefit limit.
Radiation therapy	999/12 months
Speech therapy	When preauthorization is required, up to 15 visits upon initial request. Subsequent visits in 15-visit increments up to the member's benefit limit.

Transplants/mechanical hearts	999/12 months for follow-up. Requires prior authorization.
Applied behavioral analysis therapy (ABA)	Units approved are measured in hours and defined by Clinical Review over a six-month duration when preauthorized.
Procedures	Units vary by procedure/12 months

5.1.3.4. Extensions

Requests for more visits or more time require a new request for authorization which can be submitted using the Referral Request tool.

5.1.3.5. Authorizations for Medicare Coordination of Benefits

We don't require and won't offer a prior authorization when we are secondary to Medicare (except for massage therapy and home infusion). If Medicare covers a service, we will cover as secondary. If Medicare does not cover a service, we will process the claim applying the member commercial contract and medical necessity review, if required. Requesting a prior authorization when Kaiser Permanente is secondary to Medicare is discouraged.

5.1.3.6. Medicare Coordination with a Maintenance of Benefits Plan

When a Kaiser Permanente Medicare-eligible member is not eligible for our Medicare Advantage plans, we will coordinate benefits with traditional Medicare.

When Medicare is the primary payer, you must bill Medicare directly for services through the Medicare crossover process.

Note: Do not bill Kaiser Permanente for claims that will crossover electronically. This creates duplicate billing or payment. Remember to check your Medicare explanation of payment form, Reason Code MA18.

Provider Requests for Exemption from Prior Authorization Requirements for Selected Services (PDF)

5.1.4. Radiology Services

5.1.4.1. Advanced Imaging Services

Advanced imaging includes CT, CTA, MRA, MRI, and PET services. Submit your request through the Referral Request tool.

Puget Sound region providers: All radiology requests for services at a Kaiser Permanente facility must include a completed Radiology Imaging Request form.

***Radiology requests for MRIs** must also include a completed MRI questionnaire.

Kaiser Permanente requires prior authorization for computed tomography (CT), magnetic resonance imaging (MRI), magnetic resonance angiography (MRA), and positron emission tomography (PET) services to ensure clinically appropriate imaging for our members.

Note: Claims adjudication requires prior authorization for payment. If no authorization is present, we will deny the claim to the provider's financial liability. The denial will impact the rendering provider's claims; that is, the provider providing the imaging service, not the provider ordering the service.

To expedite patient care, check the CPT/HCPC code in the PreAuthorization Code Check tool. The tool will inform you if an authorization is needed. If it is needed and there is documentation indicated for the review, attach those records to your referral request in Affiliate Link.

5.1.4.2. Imaging Services Requiring Prior Authorization

Services requiring prior authorization include but are not limited to:

- CT scans of abdomen, chest, head, lumbar spine, soft-tissue neck, facial bone, pelvis, sinus, and extremities
- MRI/MRA scans of abdomen, head, chest, cervical spine, hip, knee, lumbar spine, soft-tissue neck, shoulder, thoracic spine, and extremities
    - Positron emission tomography (PET)
    - Breast MRI

- CT colonography
- CT angiography
- Whole body CT
- Discography
- Single photon emission computed tomography (SPECT) for behavioral health indications
- MRA for behavioral health indications

5.1.4.3. Imaging Services Not Requiring Prior Authorization

Imaging services that do not require prior authorization include those ordered and provided during:

- Emergency room visits
- Hospital-based urgent care
- Hospital inpatient services
- Ambulatory surgery procedures

For questions, contact the Provider Assistance Unit (PAU) at 1-888-767-4670.

High End Imaging Site of Care Policy

5.1.5. Referring Kaiser Permanente Members for Specialty Care and Services

5.1.5.1. Specialty Care Referrals

When appropriate, you may need to refer a patient for specialty care and services. When referring a Kaiser Permanente Core member, you should always refer to a network specialist. Use the provider directory to determine who is in network for your patient's plan.

The specialist's office may need to contact our Review Services Department to obtain prior authorization for the situations listed below.

5.1.5.2. Emergency Room

If a specialist provides consultation for a member in an emergency room (in person), regardless of the provider's contract status, the

specialist may be able to see that member for appropriate follow-up care with prior authorization, if necessary.

Request for authorization of follow-up care will be reviewed on an individual basis to determine medical necessity.

An emergency room specialist may refer a patient directly to an ophthalmologist or optometrist for care, regardless of the specialist's contract status. The scope and length of that authorization will be determined by a clinical review of the request.

5.1.5.3. Inpatient Care

Contracted specialty physicians in a member's network who have consulted on a patient during an inpatient stay are allowed appropriate follow-up care with the patient in accordance with our authorization guidelines, provided that care begins within 30 days of the hospital discharge.

Non-contracted or non-network physicians who have consulted with a patient during an inpatient stay can see the patient for consultative follow-up care — for the purpose of transitioning care to a network provider — provided that care begins within 30 days of the hospital discharge.

The patient's primary care provider or contracted specialist may need to submit a request to authorize any additional visits to both contracted and non-contracted specialists.

5.1.5.4. Referring to Non-Contracted Providers

A provider may use or order services from other physician groups, facilities, or vendors for patient care. Contracted and network entities should always be used for Kaiser Permanente Core members in these instances. The use of non-contracted providers without the proper authorization from Kaiser Permanente may lead to a denial where we are unable to protect our members from balance billing or for being held financially liable for the full charge.

Examples of this may include but are not limited to:

- A lab test that is ordered and drawn at a contracted laboratory but is sent to a non-contracted pathology vendor for analysis.

- A nebulizer is given to a member from an on-site supply closet that is stocked by a non-contracted vendor.
- A member is referred by a PCP to non-contracted specialist and no preauthorization is obtained.

It is our expectation that our contracted providers work with us to ensure the best care and coverage for members.

5.1.6. Requesting Preauthorization for Coverage

Clinical specialists review the request based on clinical criteria and medical records. Supporting documentation must be submitted with the request. If the initial reviewer cannot approve it, the request is forwarded to a physician, pharmacist, or psychiatrist.

Second-level reviews:

If the first-level review results in a denial, a second-level reviewer may consult with the referring practitioner. If the second-level review is denied, a written notice of noncoverage will be mailed to the member and to the provider.

Notice or noncoverage:

This notice includes the denial rationale, review criteria, appeal instructions, and contact information for the reviewer. See Section 5.5, Member Appeals, for more information.

5.1.7. Retroactive Authorizations, Extenuating Circumstances, and Provider Referral Reconsideration Requests

Kaiser Permanente requires that providers request authorization for services prior to or within fourteen calendar days of services rendered. Exceptions to this policy are made only under standard extenuating circumstances as defined by the WorkSMART Institute's best practice recommendations (PDF), and under Washington State Administrative Code 284-43-2060.

5.1.7.1. Retroactive Authorizations

Kaiser Permanente will accept a request for retroactive authorization if the request meets either of the following guidelines:

- The request precedes a bill for services (no claim received by Kaiser Permanente) and is within fourteen days of the service OR
- The request precedes a bill for services (no claim received by Kaiser Permanente) and one of the extenuating circumstances applies

5.1.7.2. Extenuating Circumstances

If your request for retroactive authorization qualifies under the guidelines above, you may submit your request to Review Services via One Health Port, or telephone. If your request is more than fourteen days after the date of service, indicate which of the extenuating circumstances apply.

Extenuating circumstances fall into three categories:

- Unable to Know Situation-The provider and/or facility is unable to identify from which health plan to request an authorization. The patient is not able to tell the provider about their insurance coverage, or the provider verified different insurance coverage prior to rendering services.
- Not Enough Time Situations-The patient requires immediate medical services and the provider is unable to anticipate the need for a preauthorization immediately before or while performing a service.
- An enrollee is discharged from a facility and insufficient time exists for institutional or home health care services to receive approval prior to the delivery of the service.

5.1.7.3. Requirements for Retroactive Preauthorization Consideration

If the provider was unable to request prior authorization for services as required by the provider's contract and the member's coverage agreement, Kaiser Permanente will accept the request for authorization more than fourteen calendar days after services are delivered as long as the provider made the request prior to submitting the claim for payment. The provider must provide proof of the extenuating circumstance in the retroactive request for authorization. If this documentation is not provided, Kaiser Permanente will deny the request and will not reconsider the decision. Refer to the Washington Administration Simplification Guidelines for more information on extenuating circumstances and preauthorization rules.

5.1.7.4. Reconsiderations of a Denial

For reconsiderations of a denial, see Section 6.3, Provider Reconsideration Process.

5.1.8. Transplant Services

Kaiser Permanente covers medically necessary organ transplants for its members. To assure the highest quality of care and affordability for its members, Kaiser Permanente is partnering with Kaiser Permanente National Transplant Services (NTS) for all transplants except kidney transplant surgeries, which will continue to be managed within the Kaiser Permanente Washington region.

The NTS's goal is to provide members with access to a network of transplant programs located at premier medical centers known nationally for their respective transplant programs and where successful outcomes are predictably high.

5.1.8.1. Requesting a Transplant

Refer to the Transplant Process Job Aid for instructions on how to proceed with preauthorization for all transplants.

5.1.8.2. Member Benefits

Benefits may vary for member and living donor travel/lodging expenses. Members should contact Member Services at 1-888-901-4636 to inquire about their coverage.

5.2. Prior Authorization -Mental Health

5.2.1. Applied Behavioral Analysis

Kaiser Permanente provides coverage for applied behavioral analysis (ABA) treatment to individuals with autism spectrum disorder (ASD) or developmental disability for which there is evidence that ABA therapy is effective.

5.2.1.1. Coverage and Referral

ABA treatment requires a diagnosis of autism spectrum disorder or developmental disability for which there is evidence that ABA therapy is effective. A referral for ABA treatment is required from a licensed health, mental health, or allied health provider (e.g., physician, psychologist, or speech-language pathologist).

5.2.1.2. Authorization Process

Preauthorization is required for ABA treatment, regardless of patient insurance plan. Requests for ABA treatment are reviewed to determine whether they meet our ABA Clinical Review Criteria.

ABA services are delivered by providers who are contracted with Kaiser Permanente. If contracted providers are not accessible, treatment can be delivered by non-contracted provider who meets all state regulatory and licensure requirements laid out in the clinical criteria. The authorization process is as follows:

5.2.1.3. Preauthorization Process

- When a Kaiser Permanente member receives a referral for ABA services, a preauthorization request is generated to pre-approve an ABA assessment.
- There is an initial review of a referral to determine whether an enrollee meets eligibility criteria for ABA services (i.e., diagnosis, coverage, presence of autistic behaviors that are having clinically significant impact on functioning, in home, school, and/or community).
- If the enrollee meets clinical criteria for ABA services, an ABA assessment preauthorization is approved.
- If the member already has an ASD diagnosis and has the formal documentation, the ABA provider can submit a preauthorization request with the diagnosis documents.
- The letter of approval for preauthorization of services will contain the CPT/HCPC code.
- ABA assessment varies among patients; Kaiser Permanente does not issue a set number of hours for the initial assessment.
- The hours used for initial assessments can range from 6-10 hours.
- Any request that exceeds 10 hours will need to provide an additional clinical justification and hours utilization breakdown

using the Kaiser Permanente ABA PreAuthorization Request Template.

5.2.1.4. Initial ABA Authorizations

The ABA provider submits the initial individualized treatment plan (ITP) to Kaiser Permanente for clinical review (reference clinical criteria and report templates for tips on completing ITP). The ITP must use the Kaiser Permanente ABA Initial Assessment Template to ensure all information needed is received.

- If additional information is needed, the clinical review team will send the ABA provider a "records request" letter delineating the items requested as well as a due date.
- Records requests have a 3/5-day-calendar timeline from the day they are sent out (including weekends and holidays).
- It is imperative that all items requested be submitted to the clinical review department no later than the due date. Failure to comply with the deadline may result in a denial of services.
- ITPs that meet clinical criteria will be approved, and the patient and provider will receive an approval letter.
- The approval letter will list all the CPT/HCPC codes approved as well as the number of hours approved for each code.

5.2.1.5. Continued ABA treatment

- After six months, a progress report must be submitted by the ABA provider to determine whether enrollee continues to meet criteria for ABA therapy. The treatment plan will be reviewed by Kaiser Permanente and, if approved, an additional six months of ABA therapy is authorized.
- Progress reports must be submitted 15-days before the end of the current authorization period.
- Progress Reports submitted after the 15-day due date, may not be eligible for retro pay on claims that fall on days with no current authorizations.
- To account for any extenuating circumstances that prevented the ITP to be submitted on time, the ABA provider must include a detailed explanation with the report.
- Initial Treatment and Progress Plans are to be submitted to review services via Affiliate Link or fax.
- The Kaiser Permanente report templates are to be used for initial and continued care reports. Failure to use these templates may

result in submission of incomplete information which would delay the authorization process.

Kaiser Permanente ABA Progress Report Template

5.2.1.6. Termination of Services

- An individualized treatment plan is warranted when ABA services end. Use the Kaiser Permanente ABA Termination Report Template.
- Explain why the patient is discharging from the ABA provider's care.
- Include information on all progress the patient made and how the goals should be maintained after discharge.
- Explain what type of treatment the patient is stepping down to (i.e., counseling, school management, medication, etc.)

5.2.1.7. Addendums to Current Authorizations

There are times when issues arise in the middle of a six-month authorization that require a change in treatment hours. For any requests to increase or change approved ABA services, the ABA provider must submit an addendum. Use the Kaiser Permanente ABA Addendum Request Template.

5.2.1.8. Appeals

- If you disagree with a preauthorization decision, the member may appeal the decision.
- Appeals can only be initiated by the Kaiser Permanente member.
- The ABA provider can act on the members behalf with a written authorization.
- The appeal decision is made by a reviewer who is totally independent of the initial reviewer, and the decision is issued on a specified timeline.

5.2.1.9. Contact Information

Submit preauthorization requests via Affiliate Link (preferred method) or fax:

Affiliate Link User Guide – manual entry and job aid available
Once you set up your profile the authorization letter will be sent to the in-basket.

Fax: 844-660-0717
Attention: ABA Clinical Review - Elizabeth Silva-Kaplan, ABA Program Manager

Clinical Review Department
For status updates and issues with referrals
1-800-289-1363, option 2, option 4

Provider Assistance Unit
For issues with claims and patient healthcare plan questions
1-888-767-4670

Further assistance with claims:
Escalation Claim Form
For any additional information and clinical questions, contact the ABA department.

Medical offices
- Medical center hours and locations
- Holiday closures and hours

5.2.2. Coordinating Mental Health Inpatient or Residential Care

When treating members in mental health inpatient or residential settings:
- All services must be preauthorized by the Mental Health Access Center (MHAC) at 206-901-6300 or 1-888-287-2680.
- Authorizations are not provided after-hours, on holidays, or on weekends. In these cases, contact EPRO (Emergency Patient Resources and Options) at 1-800-337-3197 to inform Kaiser Permanente of admission. The MHAC will review on the next business day.
- Throughout the member's stay, we will review their care for appropriateness of placement and participate in discharge planning.
- To arrange follow-up care, contact the MHAC prior to discharge. Follow-up care must be provided within seven days of the discharge date.

If a member needs to transfer from a mental health facility to a medical or surgical facility, notify Kaiser Permanente via EPRO at 1-800-337-3197 and the member's primary care provider.

We reserve the right, upon consultation with a Kaiser Permanente physician, to transfer any member to a Kaiser Permanente facility, as stated in the member's medical coverage agreement. If the member refuses to transfer to

one of our facilities, costs incurred during the hospitalization may be the member's sole responsibility.

5.2.3. Excluded Mental Health Services and Addiction and Recovery Service

The following services may not meet the definition of clinical necessity. Check a member's eligibility before proceeding with care.

5.2.3.1. Mental Health and Wellness Excluded Services

Any services deemed not medically necessary by Kaiser Permanente will be excluded from coverage. We may exclude the following services from coverage depending on the member's health plan. Below are the most common exclusions in many of the plans covered by Kaiser Permanente. For specific patients, contact Member Services or the Mental Health Access Center.

- Assessment and treatment services primarily vocational and academic in nature (such as educational testing, sensitivity training, etc.) if not medically necessary.
- Treatment for conditions where improvement or stabilization cannot be reasonably expected per the medically necessary care definition.
- Evaluation or treatment mandated by a third-party unless considered medically necessary (for example, court, employer, or school).
- Documentation in the form of reports or summaries of clinical information.
- Long-Term and Custodial care.
- Experimental or investigational therapies.
- Educational programs (for example, experiential programs such as Wilderness Therapy or Boarding School programs).
- Nicotine-related disorders.
- Treatment specific to and solely for learning, intellectual disabilities, and academic or career counseling.
- Treatment specific to and solely for personal growth or relationship enhancement.
- Work or school ordered assessment and treatment not determined to be medically necessary.
- Services solely for marital counseling, parent/child relationship counseling, or other relational counseling services that do not meet mental health clinical medical necessity criteria.

- Wilderness therapy or boarding school that does not meet approved criteria for mental health residential level of care.
- Genetic testing or genetic sequencing solely for psychotropic treatment and/or management that does not currently meet mental health medical necessity criteria due to unclear guidance of how test results should be used to direct treatment decisions and insufficient evidence of improved patient health outcomes.
- Kaiser Permanente will only approve treatment with providers/facilities that are in alignment with treatment standards that employ evidence-based programs and practices. Kaiser Permanente will not authorize treatment that utilizes experimental or investigational therapies (e.g. Aversion Therapy and Conversion Therapy).
- Any other service not specifically listed as covered or excluded in the member's benefit contract and does not meet medical necessity criteria and/or is not approved by Kaiser Permanente as an evidence-based treatment in alignment with the Kaiser Permanente treatment philosophy.

5.2.3.2. Addiction and Recovery Excluded Services

Members must have a current diagnosis and symptoms of a substance use disorder as defined by the current Diagnostic and Statistical Manual of Mental Disorders (e.g. DSM V) to be eligible for treatment services, with symptoms significantly interfering with the individual's ability to function in at least one life area.

There must be a reasonable expectation that the patient can make changes resulting from the proposed treatment and that stabilization is possible.

The proposed medical treatment must involve a level of care with appropriate resources to assess and treat the client's condition according to its severity and the consumer's health and level of functioning.

The proposed medical treatment must involve the least intensive level of care necessary to accomplish the treatment objectives in a clinically appropriate manner.

Exclusions exist in medical coverage agreements. In addition to contractual exclusions, additional exclusions may include:

- Court, school, or work-ordered treatment that is not medically necessary.
- Educational and early intervention substance abuse programs, such as Alcohol Drug Information School (ADIS) (American Society of Addiction Medicine, ASAM, level 0.5).
- Urinalysis for drug screens, unless medically necessary, authorized by Kaiser Permanente, or both.
- Halfway houses.
- Wilderness therapy or boarding school programs that do not meet approved criteria for substance use disorder residential level of care.
- Genetic testing or genetic sequencing solely for psychotropic treatment and/or management that does not currently meet mental health medical necessity criteria due to unclear guidance of how test results should be used to direct treatment decisions and insufficient evidence of improved patient health outcomes.
- Kaiser Permanente will only approve treatment with providers/facilities that are in alignment with treatment standards that employ evidence-based programs and practices. Kaiser Permanente will not authorize treatment that utilizes experimental or investigational therapies (e.g. Aversion Therapy and Conversion Therapy).

5.2.4. Inpatient Addiction and Recovery Treatment

Per ESHB 2642, the following guidelines apply to acute inpatient and residential treatment for addiction and recovery:

- Regarding withdrawal management, members receiving treatment in Washington state are eligible for 3 covered days of treatment without prior authorization. After the initial 3 covered days, ongoing care is subject to medical necessity criteria. If a member is admitted to acute withdrawal management, the treating facility must call the Kaiser Permanente Emergency Notification Line within 24 hours following the admission or as soon as medically possible. For treatment outside of the Kaiser Permanente Washington service area, prior authorization is required.
- Regarding residential substance use disorder treatment, members receiving treatment in Washington state are eligible for 2 covered days of treatment without prior authorization. After the initial 2 covered days, ongoing care is subject to medical necessity criteria. If a member is admitted, the treating facility must call the Kaiser Permanente Emergency Notification Line within 24 hours following the admission or as soon as medically possible. While members with an out-of-network benefit may

choose from which facility to receive service, treatment outside of the Kaiser Permanente Washington service area will require prior authorization.
- We will continue to cover only medically necessary residential treatment for substance use disorders for people with comprehensive benefits for substance abuse services, following prior authorization.
- While we do not require prior authorization for PPO health plan members, we do require proof of medical necessity. A provider or member may request a benefit advisory. To ensure coverage eligibility, members may request prior authorization before entering treatment.

5.2.5. Inpatient Mental Health Treatment

Kaiser Permanente requires prior authorization of inpatient mental health care for all members, including members using their out-of-network benefits. Prior authorization assures Kaiser Permanente that the services are medically necessary and appropriate, that the provider and/or facility is fully licensed and capable of providing the highest level of care and allows Kaiser Permanente to plan for adequate follow-up care.

Prior authorization is not required for admission for emergency care. However, the care must meet medical necessity criteria. If a member is admitted to inpatient treatment as a direct result of an emergency room visit, the member or a family member must call the Kaiser Permanente Emergency Notification Line within 24 hours following the emergency or as soon as medically possible. You can obtain authorization after the health plan reviews clinical information, normally provided by the treating provider.

Kaiser Permanente complies with WA E2SHB 1688 related to coverage of emergency services and alignment of state and federal balance billing laws.

5.2.6. Medically Necessary Mental Health Services

All mental health clinical services must be medically necessary to be covered by Kaiser Permanente. The overall definition of medical necessity for mental health care is consistent with the definition of medical necessity as provided by the organization, which is as follows:

Medically necessary mental health services are defined in manner consistent with other medical services if recommended by the enrollee's treating provider and determined to meet medical necessity by Kaiser Permanente's medical director of the Mental Health Access Center, or his or

her designee, according to generally accepted principles and standards of quality, evidence-based medical/psychiatric practice and Kaiser Permanente's approved medical necessity criteria, which are rendered to an enrollee for the diagnosis, care, or treatment of a formal diagnosis based on current Diagnostic Statistical Manual (e.g. DSM V) diagnostic criteria. To be medically necessary, services and supplies must meet the following requirements:

- Are not solely for the convenience of the enrollee, his or her family, or the provider of the services or supplies.
- Are the most appropriate level of service or supply which can be safely provided to the enrollee.
- Are for the diagnosis or treatment of an actual or existing mental health condition or substance use disorder as defined by the current Diagnostic Statistical Manual for psychiatric conditions (e.g. DSM V) – see below – unless being provided under Kaiser Permanente's schedule for preventive services.
- Are not for recreational, life enhancing, relaxation, or palliative therapy, except for treatment of terminal conditions.
- Are appropriate and consistent with the diagnosis and which, in accordance with accepted medical standards in the state of Washington, could not have been omitted without adversely affecting the enrollee's condition or the quality of health services rendered.
- As to inpatient care (whether inpatient hospital or residential level of care), could not have been provided in a provider's office, the outpatient department of a hospital, or a non-residential facility without affecting the enrollee's condition or quality of health services rendered.
- Are not primarily for research and data accumulation.
- Are not experimental or investigational services.

The length and type of the treatment program and the frequency and modality of visits covered shall be determined by Kaiser Permanente's medical director of mental health utilization review/utilization management, or his or her designee.

5.3. Clinical Review Criteria

Review Kaiser Permanente's Clinical Review Criteria on our provider website.

5.4. Prior Authorization and Utilization Review

5.4.1. Routine Prior Authorization Reviews

When you submit a prior authorization request, Review Services will either approve or deny the request. If denied, you can appeal on your patient's behalf.

5.4.1.1. First-Level Reviews

Clinical specialists review the request based on clinical criteria and medical records. Supporting documentation must be submitted with the request. If the initial reviewer cannot approve it, the request is forwarded to a physician, pharmacist, or psychiatrist.

5.4.1.2. Second-Level Reviews

If the first-level review results in a denial, a second-level reviewer may consult with the referring practitioner. If the second-level review is denied, a written notice of noncoverage will be mailed to the member and to the provider.

5.4.1.3. Notice Of Noncoverage

This notice includes the denial rationale, review criteria, appeal instructions, and contact information for the reviewer.

5.4.2. Urgent Prior Authorization Reviews

Requests identified as "Urgent/Expedited" must meet the following definition:

- An urgent care review request is for approval of care or treatment that, if delayed, could seriously jeopardize the patient's life, health, ability to regain maximum function, or cause severe pain that cannot be managed without the requested care.
- Requests that are identified as "Urgent" that do not meet this definition cause unnecessary and possibly harmful delays in responding to other urgent requests.

5.4.3. Utilization Reviews

Our goal is to provide high-quality care in the appropriate setting, at the right time, and by the most appropriate practitioner. Care management decision making is based only on the appropriateness of care, service, and the existence of coverage.

- Utilization management involves reviewing requests for coverage based on need and discussing coverage determinations.
- Staff are available at 1-800-289-1363 on weekdays from 8:30 a.m. to 4:30 p.m. PST to answer questions about utilization review decisions.

5.5. Member Appeals

Members who receive a denial, reduction, or limitation notice for services may initiate an appeal within the specified timeframe as described in their notice of non-coverage or explanation of benefits. All coverage determinations that result in a denial, reduction in coverage request, or a failure to provide or make payment (in part or in whole) for a benefit, contain instructions for a member appeal.

5.5.1. Standard Appeals

5.5.1.1. Non-Medicare Advantage Appeals

Standard non-Medicare Advantage appeals can be submitted orally or in writing. You may request a standard appeal on the member's behalf when the member has signed the appropriate Authorization of Representative (AOR) form, found on the Forms page on our provider website. You may supply supporting statements and records in a member's appeal. We resolve most non-Medicare Advantage standard appeals within 14 to 30 days.

If you are the treating provider, you may request an expedited appeal (72-hour completion) without member permission when the standard appeal timelines (7 to 60 days) meet one of the following criteria:

- Could seriously jeopardize the life or health of the member
- Could seriously jeopardize the ability of the member to regain maximum function
- Delay would subject the member to severe pain that could not be adequately managed without the care or treatment

If you submit an expedited request, include medical records to support your appeal and provide a valid phone number should we need to contact you for any reason.

5.5.1.2. Medicare Appeals

Standard Medicare Advantage appeals must be in writing. For standard pre-service Medicare Advantage appeals, with the member's permission, you may file an appeal on their behalf without an AOR. You may supply supporting statements and records in a member's appeal. Standard appeals timeframes are as follows:

Medicare Advantage pre-service appeals:
Part B – 7 days
Part C – 30 days
Part D – 7 days

Medicare Advantage post-service appeals:
Part C – 60 days
Part D – 14 days

5.5.2. Expedited Appeals

If a member's life, health, or ability to regain maximum function would be jeopardized by following the standard appeal process, you may request an expedited appeal on their behalf without an AOR. You must make your request to Kaiser Permanente orally or in writing before the member receives the services in question.

Appeals criteria and time frames for resolution are identical for all members requesting an expedited appeal. Since our response time is 72 hours or less, provide medical records to support the appeal within 24 hours or less of the request, using fax, courier, or other appropriate means.

We respond to Medicare Advantage and non-Medicare Advantage expedited appeals within 72 hours from their receipt. However, some non-Medicare Advantage contracts specify different time frames and appeal processes.

We handle requests that do not meet the expedited appeal criteria according to our standard 7-60-day redetermination/reconsideration (appeal) processes.

5.5.3. Subsequent Appeal Levels

Subsequent appeal levels vary depending on the member's health plan group. If a denial letter is issued, a member may request an external review. These processes are described in detail in the appeal attachments sent with the denial or appeal letters.

5.5.4. Services Provided During Appeal

If a practitioner provides services to an enrollee during an appeal or adverse certification process, the practitioner must provide written notification to the enrollee that payment for services is the enrollee's responsibility, unless the carrier elects to pay.

6. BILLING AND CLAIMS

6.1. Payment Policies

View our Payment Policies on the Kaiser Permanente provider website.

6.2. Claims Submission and Processing

6.2.1. Checking Claims Status

You can check claim status using our Claims Status Inquiry tool regardless of whether the claim was submitted electronically or on paper. You also can review an electronic file from your billing service or clearinghouse for all pending claims or contact the Provider Assistance Unit at 1-888-767-4670.

If you have checked the status of a claim submitted to Kaiser Permanente and we do not show it in our system, the following steps will help you locate your claim.

6.2.1.1. Paper Claims

Paper claims sent within the last week may not be entered in our system yet. You can resubmit a claim if it has been two weeks or more since you submitted it and you have verified that it is not in our system.

6.2.1.2. Electronic Claims

If you submit claims electronically using a billing service or clearinghouse, check the report for rejected claims caused by data submission errors. If errors are found, contact the service or clearinghouse for resolution. Refer to "Electronic Segment Information" in the CMS-1500 or UB-04 required fields forms to assist you in your discussion with the clearinghouse or billing service.

The member's name must be exactly the way it appears on the member's ID card and in our system, including spaces and punctuation.

If you have electronic confirmation that Kaiser Permanente received the claim, contact the Provider Assistance Unit to initiate an investigation. Be ready to provide the following information:

- Provider tax ID number
- Kaiser Permanente member ID number
- Date of service
- Total charges submitted
- NPI number
- The name and contact information of your billing service or clearinghouse
- Type of claim (professional or institutional)
- Date and time of file submittal or expected transmittal
- Whether the missing claims are in a series or just a few out of a batch
- Start/end or pattern of problems
- Patient account number
- Contact name and phone number

6.2.2. Claims Adjustments

If you wish to request an adjustment to a claim that you think was incorrectly processed, see Section 6.3, Provider Reconsideration Process, for more information.

If the adjustment involves an overpayment by Kaiser Permanente, the overpayment may be available immediately to offset future reimbursements.

Adjustments
We may also initiate adjustments if we determine that we processed a claim incorrectly or as a part of a reconciliation.

The adjustment process involves:

- Cancelling the original payment and creating a credit balance to the provider.
- Processing the claim a second time to pay the correct amount or to deny the claim, whichever is appropriate.

When the adjustment results in additional money owed to you, you will be paid in one of three ways (depending upon the amount of your current credit balance):

1. Issuing a check.
2. Using money from outstanding credit balance to pay the current claim (offset process).

3. Using a combination of money by a check and application of outstanding credit balance to pay the claim (check and offset process).

If it results in overpayment, we will create a credit balance to apply to a future claim payment. If Kaiser Permanente initiated overpayment, RCW 48.43.600 will apply.

There are some instances in which a credit balance may not be resolved by applying to a future claim. When overpayments are not paid back by future claim payment, you will need to send a check to pay off the overpayment.

Note:
- Credit balances created by self-funded employer group claims can only be applied to future claims from the same employer group.
- Credit balances created by FEP claim can only be resolved by applying to future FEP claims.

Initiating an adjustment
See Section 6.2.22, Timely Filing of Claims, for more information.

Contesting an adjustment
If a contracted non-facility provider fails to contest the request in writing within 30 days of the provider's receipt of the request, we will deem the request accepted and will proceed with the adjustment process.

If a contracted non-facility provider contests the refund request, we may not require payment of the disputed adjustment amount any sooner than six months from the date of the original notice from the carrier. However, nothing precludes you from permitting us to apply the offset at an earlier date.

Notices regarding adjustments should be sent to:

Kaiser Permanente Claims Administration
Attn: Claims Reconsideration
P.O. Box 30766
Salt Lake City, UT 84130-0766

6.2.3. Claims Explanation of Payment/835 Remittance Advice & Reimbursement

You can receive your 835 remittance advice (RA) weekly by electronic batch transaction with remittance information auto-posted to patient accounts or by paper Explanation of Payment (EOP) mailed to your office. Providers not electing electronic funds transfer will receive a check via U.S. Postal Service.

6.2.4. Caring for Kaiser Permanente Members from Other Regions

Members of other Kaiser Permanente regions may receive care in any of our clinics or by one of our contracted providers in the Kaiser Permanente Washington region. Visiting members are subject to the same business practices and prior authorization rules as in the Kaiser Permanente Washington Core plan.

Please contact the Visiting Member Unit toll-free at 1-800-446-4296 or 509-241-7798 for instructions when seeing a visiting Kaiser Permanente member from another region. For prior-authorization questions, contact Review Services at 1-800-289-1363.

Please be prepared to furnish the following information:

- Member's last name, first name, middle initial
- Date of birth
- Kaiser Permanente HOME plan name and ID number
- Social Security Number

Please refer to the Visiting Member Guidelines for instructions to verify eligibility, benefits, authorizations and claims processing

Note: For authorizations for the following services, please contact the member's HOME plan:

- Infertility services
- Services related to artificial conception
- Gender confirming surgery and related services, other than services determined to be provided by all regions
- Services related to bariatric surgery and treatment
- Organ and blood/marrow transplants and related care
- Orthotics and prosthetics
- Durable Medical Equipment (DME)
- Chronic dialysis

6.2.5. Claims Submission Requirements

The Claims Submission Standards and Specification have been updated. For specific information on requirements and recommendations, please see the Paper Claim Submission Standards and Specifications Grid.

Three essential components must be included with each submitted claim:

- **Patient and subscriber information:** Include the patient's Kaiser Permanente member ID number and name as it appears on their ID card and your patient identification information. Include other insurance or subscriber information if applicable.
- **Provider information:** To avoid an error or delay in claims payment, notify Provider Contracting or Provider Services as early as possible of any changes to your tax ID number (TIN), address, name, or to how you use or designate your entity National Provider Identifier (NPI).
- **Coding:** We expect all claims to be submitted with industry-standard coding and that procedures be reported with the HCPCS/CPT codes that most comprehensively describe the services performed. For more information, see our Code Editing and Coding Policy and Coding Guidelines for Preventive Care Services Commercial (Non-Medicare) Kaiser Permanente Plans

The Kaiser Permanente Payor ID is **91051**
The Health Plan Services Payor ID is **94320**

6.2.6. CMS-1500 Claims — Coding for Services Provided

Ambulatory surgical centers
When billing for facility services on a CMS-1500 claim form, modifier SG must be billed on the service line(s) in order to identify the claim as a facility charge.

Code editing and review process
Most health plans, including Kaiser Permanente, have code editing processes in place to assure that claims are coded based on industry standard guidelines. Kaiser Permanente has adopted the best practice recommendation for Claim Coding Policy and Edits: Standardization & Transparency as outlined in Washington state Senate Bill 5346 and published through the administration simplification work of Washington Healthcare Forum.

Kaiser Permanente may, on occasion, either through the code editing process or claims audit process, review a provider's claims in more detail. It may be necessary for us to request the medical records documentation which support the services billed in order to complete these audits. Examples of coding reviews that might prompt a request for medical records documentation: codes with modifier 22, codes with modifier 59, "unlisted" codes, and high-level E&M codes.

6.2.7. Coordination of Benefits

To comply with Washington State Office of the Insurance Commissioner regulations, health plan carriers coordinate benefits so that when an individual has more than one Kaiser Permanente plan, each plan pays its share of the medical expenses.

If your patient is covered by two or more health plans, we can coordinate benefits with the other health plans to help the patient receive the full benefit of those plans. By coordinating benefits, we may be able to waive or reduce out-of-pocket expenses for covered services.

Dual Kaiser Permanente plans
When caring for a Kaiser Permanente member with dual Kaiser Permanente plans, identify the primary health plan following Washington Administrative Code Standards for Coordination of Benefits (WAC 284-51). If you are unsure which health plan is the primary health plan, contact the Provider Assistance Unit at 1-888-767-4670.

Kaiser Permanente as secondary carrier
If Kaiser Permanente is the secondary to another health plan, submit the claim to the primary health plan first. All Kaiser Permanente coverage and criteria requirements apply, regardless of whether Kaiser Permanente is the primary or secondary health plan.

When a member has primary insurance, the secondary plan may be required to waive the gatekeeper requirement when the primary plan's requirements have been met.

For FEHB Core and Self-Funded Core plans that follow the maintenance of benefits rule, the amount paid by the primary insurance is used to reduce the amount that Kaiser Permanente would pay as the secondary. Members must follow the network and authorization rules of their Kaiser Permanente plan for claim payment, even if it is not the primary plan. Members will be responsible for cost-shares determined by the plan.

6.2.8. Copayments

Most Kaiser Permanente plans require members to make copayments directly to the provider. Copayments for office visits, pharmacy services, emergency room care, and inpatient hospital care can be found online if you are registered through OneHealthPort to access Kaiser Permanente.

For members on Medicare hospice plans or contracts, copayment applies to non-hospice services only.

You should collect copayments from our members at the time service is provided. You may charge interest, a reasonable billing fee, or both on unpaid copayments as stated in your office policy. The exceptions are Medicare and Medicaid enrollees, for whom it is against federal regulation to collect such fees.

You should collect copayments for office visits only when the member sees a physician, physician's assistant, or nurse practitioner. There is no copay for seeing a lab technician.

Some group plans cover preventive care visits in full. If a member is on such a plan, do not collect a copayment.

Contact the Provider Assistance Unit (PAU) at 1-888-767-4670 with questions about copayments.

6.2.9. Deductibles and Coinsurance

Some health plans require deductibles, coinsurance, or both. In both cases:

- The member is responsible for paying any applicable deductible or coinsurance.
- You may bill for deductibles and coinsurance after you receive a remittance statement explaining how much to collect from the member.

While a Medicare hospice election is in effect, we will pick up the covered balances (deductible or coinsurance) after Medicare makes payment for non-hospice related services.

6.2.10. Electronic Batch Transactions

Kaiser Permanente uses the following:

Direct
Authorized users can sign on via OneHealthPort to:

- Monitor the status of claims
- Manage referrals
- Check member eligibility

Electronic funds transfer/electronic remittance advice (EFT/ERA)
Kaiser Permanente offers EFT/ERA through our business partner, U.S. Bank. Electronic Funds Transfer/Electronic Remittance Advice (EFT/ERA)

Clearinghouse/direct connections
Business-to-business electronic data interchange (EDI) to facilitate our claims processing, patient billing, and care management systems. Clearinghouse/Direct Connections

Rebilling
If there are no changes to a claim, do not rebill until you have confirmed that we have not received your claim by using either our Claims Status Inquiry tool or contacting the Provider Assistance Unit at 1-888-767-4670.

6.2.11. Electronic Funds Transfer and Electronic Remittance Advice

Kaiser Permanente uses electronic fund transfer (EFT) and electronic remittance advice (ERA) for its business services. EFT/ERA results in faster payment, reduced administrative costs, and increased efficiency. Providers currently receiving ERA through a clearinghouse can continue to do so, and sign up for EFT to receive additional benefits.

- Get paid faster and reduce costs by eliminating paper checks and remittance advice by receiving free ERA/EFT with U.S. Bank Payment Accelerator, powered by InstaMed.
- Get paid faster with claims payments directly deposited into your bank account.
- Automate payment posting to streamline processes.
- Automate reconciliation of payments and remittances.
- Access reporting 24/7 to simplify your workflow.

Enroll with U.S. Bank payment accelerator

1. Enroll online and start receiving benefits immediately
2. Download and complete the network funding agreement from U.S. Bank
3. Attach a voided check
4. Fax or mail the form to U.S. Bank
   Toll-free fax: 1-877-755-3392
   Mail to:
   U.S. Bank Payment Accelerator (c/o InstaMed)
   P.O. Box 58790
   Philadelphia, PA 19102

Updates to existing enrollment
To add or change Tax Identification or NPI numbers already enrolled with InstaMed for electronic funds transfer, please do the following:

1. Email support@instamed.com with the updated information.
2. Notify Kaiser Permanente of these changes by completing an External Demographic Form.

Questions?
For questions about the enrollment process, contact the U.S. Bank Payment accelerator team toll-free at 1-877-833-6821, or send an email to connect@instamed.com.

For more information about EFT/ERA, see Electronic funds transfer and remittance advice FAQs and EDI and HIPAA: Frequently asked questions.

6.2.12. Medicare Coordination of Benefits

When a Kaiser Permanente Medicare-eligible member is not eligible for our Medicare Advantage plans, we will coordinate benefits with traditional Medicare.

When Medicare is the primary payer, you must bill Medicare directly for services through the Medicare crossover process.

Note: Do not bill Kaiser Permanente for claims that will crossover electronically. This creates duplicate billing or payment. Remember to check your Medicare explanation of payment form, Reason Code MA18.

When Kaiser Permanente has referred the service, we will provide payment for all covered balances.

Medicare effective in the middle of inpatient stay
When Medicare becomes effective and Primary in the middle of an inpatient confinement, providers are not allowed to bill any other parties for dates preceding entitlement.

Medicare will pay for all the claims as outlined in the Medicare Claim Processing Manual, Publication 100-04, Chapter 3, Section 40.

6.2.13. Members' Financial Responsibilities

A member's financial responsibilities (cost shares) vary depending on the specific plan benefits, copayments, coinsurance, and deductibles.

You can check member eligibility using our Eligibility Inquiry tool. After your search, scroll to the bottom of the Eligibility Inquiry Results page and click Summary of Benefits. You can also check a member's eligibility by calling the Provider Assistance Unit at 1-888-767-4670.

Once we process your claim, you must bill the member for their share of the bill only, as stated on the Remittance Advice you receive with your payment.

6.2.14. Motor Vehicle Accidents

When a Kaiser Permanente member is receiving care for injuries related to a motor vehicle accident, you should bill claims directly to the member's automobile insurance carrier under their Personal Injury Protection (PIP) if available.

The use of box 10B marked yes on a CMS 1500 form or FL31 coded with 01, 02, or 03 on a CMS 1450 form identifies an auto accident. If Kaiser Permanente does not know about the accident, we will investigate and notify you if an auto insurance carrier should be primary.

The member must stay within our network system to ensure coverage by Kaiser Permanente in case the carrier rejects the claim.

If the carrier denies the claim, promptly submit the claim and a copy of the denial to Kaiser Permanente. We will only consider claims filed within the timely filing limit of one year from the date of service.

In the absence of first-party coverage, submit all bills to Kaiser Permanente for consideration. **You should not bill any third-party carrier for care you provide related to a motor vehicle accident.**

If you have questions about a first- or third-party accident, contact our Other Party Liability Department at 1-866-783-9594.

6.2.15. NPI (National Provider Identifier) and Taxonomy

To improve payment accuracy and timeliness, bill with National Provider Identifier (NPIs) numbers and taxonomy codes wherever and whenever they apply.

All providers
At a minimum, include the following information on both electronic (837i) and paper (UB-04) claims:

- Billing provider NPI, the minimum information needed to process a claim
- Pay-to provider NPI, if different from the billing provider
- Service location NPI, if different from the billing address

Mental health
Mental health providers must include the following taxonomy code information on both electronic (837p) and paper (CMS-1500) claims:

- Billing provider taxonomy code: To indicate the type of service provided by the billing entity. Allows us to pay providers more accurately and to better track these limited benefits for our patients.
- Pay-to provider taxonomy code: Required if different from the billing provider.
- Rendering provider taxonomy code: Required if different from the billing providers.

Hospitals
In addition to the information applicable to all providers, hospitals are asked to include the following information on both electronic and paper claims:

- Billing provider taxonomy code, a combination of tax ID, NPI, and taxonomy code that replaces the Medicare facility number for hospital payment methodology
- Pay-to provider taxonomy code, if different from the billing provider

Kaiser Permanente's hospital billing requirements are aligned with those of Medicare. Like Medicare, we will recognize the following taxonomy codes for hospital units: Search NPI registry for providers and facilities

Additional taxonomy codes and information can be found on the Washington Publishing Company website.

6.2.16. Outpatient Prospective Payment System/Ambulatory Payment Classifications

Kaiser Permanente adheres to the Center for Medicare & Medicaid Services (CMS) outpatient prospective payment system/ambulatory payment classifications (OPPS/APC) for outpatient facility service billing requirements. We ask that hospitals submit OPPS/APC data for non-Medicare as well as Medicare claims.

We require that you bill modifier SG for facility charges on your CMS-1500 forms, even though CMS no longer requires it.

We must capture the data requirements listed in NPI and Taxonomy Requirements and General Medicare Data Requirements to meet CMS encounter data requirements. Please refer to UB-04 (CMS-1450) Field Descriptions for field designations.

6.2.17. Paper Claims

We enforce our policy of not accepting paper claim forms that are completely handwritten or have any handwritten notations, stamps, or stickers on them. Some exceptions apply — see the Paper Claim Submission Standards and Specifications Grid.

Any claims outside this policy will be rejected and returned to the provider with a letter of explanation.

Claims are generally processed within 30 days after receipt. You may check receipt status by contacting our Provider Assistance Unit at 1-888-767-4670 or by logging into the Claim Status Inquiry tool.

If you cannot submit claims electronically, send all paper claims to:

Kaiser Permanente Claims Administration
P.O. Box 30766
Salt Lake City, UT 84130-0766

6.2.18. Physician Reimbursement for Medical (Non-Psychiatric), Surgical, and Anesthesia Services

Kaiser Permanente calculates allowable reimbursement at the lower of charges or the maximum amount allowable on the pertinent Kaiser Permanente Washington provider reimbursement schedule described below. Paid amounts shall reflect the maximum allowable amount less any applicable coinsurance, copayments, and deductible amounts.

Medicare services
The Kaiser Permanente provider reimbursement schedule for services provided to Medicare members will generally reflect prevailing Medicare payment levels as they are revised in accordance with the Centers for Medicare and Medicaid Services'(CMS) fee schedule and payment methodology.

Commercial services
The Kaiser Permanente Washington provider reimbursement schedule for services provided to commercial members is generally based on the CMS Resource-Based Relative Value Scale (RBRVS) relative value units (RVUs), payment policies and methodology.

Final RVU values reflect adjustments for:

- Geographic adjustments (GPCI), using values for King County and "Rest of Washington" Non-Facility Practice and Facility Practice
- Expense RVU differential for site-of-service

The final RVU calculation is rounded to two decimal places.

Where CMS does not provide an RBRVS-based RVU, gap filler methodology is applied. When no gap-filler is available, Kaiser Permanente Washington will price the service at a percent of the billed charges until CMS assigns an RVU or the gap-filler methodology provides a value. Codes that are considered "by report" will be reviewed for medical necessity and priced accordingly if appropriate.

Kaiser Permanente Washington updates its professional provider reimbursement schedule periodically by adding new codes and deleting retired codes. In most instances, RVUs are assigned to new and revised codes in accordance with CMS geographically adjusted RBRVS schedule, other CMS schedules, and gap filler methodology.

Discrete facility charges for evaluation and management services provided to commercial managed-care members are not reimbursable, nor is the member liable for these charges. The Kaiser Permanente provider reimbursement schedule utilizes the prevailing Medicare Part B Drug Payment Allowance Pricing File, updated quarterly, for drugs furnished incident to physician services.

The Kaiser Permanente Washington provider reimbursement schedule utilizes the American Society of Anesthesiologists (ASA) base units and 4 time units per hour in calculating reimbursement for anesthesia services billed with base units and time. Time units are rounded to the first decimal place. For neuraxial labor anesthesia, 4 units are assigned to the first hour and 1 unit for each additional hour or partial hour. Relative value units for procedures and pain management services provided by anesthesiologists are derived from the RBRVS system described above. Please consult your contract for specific exceptions to the 4-unit standard.

Facility charges for evaluation and management services
Discrete facility charges for evaluation and management services provided to commercial managed-care members are not reimbursable. Such services, when provided to Medicare managed-care members, shall continue to follow Medicare payment rules.

1. Evaluation and management services are defined as Current Procedural Terminology codes 99201-99215, 99381-99395, 99401-99429, 99495-99496, 99078, G0463, payment for which is allowed only when billed on a CMS-1500.
2. Discrete facility charges associated with evaluation and management services are not reimbursable for commercial managed-care members, regardless of revenue code billed. Commercial managed-care members shall not be liable for these charges.
3. Professional services for evaluation and management services are reimbursed under the Kaiser Permanente Washington provider reimbursement schedule.

For more information about facility charges for evaluation and management services, see Kaiser Permanente Payment Policies.

6.2.19. Requests for Additional Claims Documentation

We developed the following procedures to help facilitate the process of sending and matching hard copy documentation requested for certain electronic or paper claims when additional information is needed.

Send reports or additional documentation only when requested.

Send any requested documentation to the appropriate address listed on the Claims Supporting Documentation Form.

Completely fill out the claims supporting documentation form. You must include the claim number, if available, so we can successfully match any new documentation with the appropriate claim.

6.2.20. Submission of Claim Supporting Documents

EXAMPLES

- Primary Explanation of Benefits
- Non-Pre-Payment Chart Notes
- Authorization to Release Medical Records

Note: Does not include all; purpose is for example only.

WHERE TO SEND YOUR RECORDS

- Electronic Submission Form: Claims Supporting Documentation Form
- Mail to:
  Kaiser Permanente
  P.O. Box 30766
  Salt Lake City, UT 98140

6.2.21. Submitting Secondary Claims

Kaiser Permanente, along with other payers participating in the Washington Healthcare Forum, has agreed to accept secondary claims electronically with no explanation of benefits (EOB) as long as the appropriate electronic fields are populated. See the Worksmart Institute's Claims Processing Policies and Guidelines for how to submit electronic and paper claims.

Participating health plans will not require providers to submit paper EOBs with electronic secondary claims as long as the primary payer is a commercial insurance company and necessary EOB information is included with the claim.

With the exception of Medicare COB, Kaiser Permanente also will accept UB-04 and CMS 1500 paper claims without a paper EOB if these guidelines are followed in completing the paper claim.

Required claim fields for secondary billing
If you leave any of the following fields empty or blank on your claim, we will deny the claim.

Form locator (FL) for CMS-1500 claims
- 9, 9a, and 9d: Other Insured Information — insurance name, identifying number, and payer ID
- 10a and 10b: Auto accidents, personal injuries, and employment-related injuries
- 19: **Note** - Must contain COBZ or COB = Z if the primary health plan paid zero
- 21: E code to highest level of specificity describing the accident or injury in 10a and 10b
- 29: Must contain a dollar amount or zero
- 33a: Billing provider NPI field

Form locator (FL) for UB-04 claims
- 32: Auto accidents, personal injuries, and employment-related injuries
- 50a, 50b, or 50c: Payer name — primary, secondary, and tertiary
- 54a, 54b, or 54c: Must contain a dollar amount or zero
- 56: Pay-to provider NPI
- 58a, 58b, or 58c: Insured's name — required if other insurance or Medicare secondary payer
- 60a, 60b, or 60c: Insured's unique ID: Kaiser Permanente member ID number
- 77: E code to highest level of specificity describing accident
- 80: **Note** - Must contain COBZ or COB = Z if the primary health plan paid zero
- 81a-d: Billing Provider and miscellaneous NPI and taxonomy fields

6.2.22. Timely Filing of Claims

To expedite billing and claims processing, we ask that your claims be sent to Kaiser Permanente within 30 days of providing the service. The Washington State Office of the Insurance Commissioner (OIC) requires health carriers to meet specific claims-payment timeliness standards. Carriers failing to achieve those standards must pay interest to contracted providers. We exceed OIC standards with our policy of paying interest on any clean claim that is not paid within 60 days of receipt.

Clean claims

The OIC defines a clean claim as "having no defect or impropriety, does not lack any required substantiating documentation, or does not have any particular circumstances requiring special treatment that prevents timely payment."

The Centers for Medicare and Medicaid Services (CMS) defines a clean claim as "a claim that has no defect or impropriety, including lack of required substantiating documentation for non-contracting providers and suppliers, or particular circumstances requiring special treatment or that prevents timely payment from being made on the claim. The claim must include information necessary for purposes of encounter data requirements."

Filing guidelines by health plan

We follow both the CMS guidelines and the Revised Code of Washington (RCW) for timely filing of original and adjustment claims. These requirements are outlined here by type of health plan. To expedite claims processing, we encourage you to submit claims using electronic transactions.

Medicaid

Original claims: The claim must be received by Kaiser Permanente within 12 months from the date of services.

Adjustment requests: The request must be received within 24 months from the date the claim was processed (RCW 48.43.605).

The Patient Protection and Affordable Care Act requires that Medicare and Medicaid overpayments be reported and returned within 60 days after they are identified by the provider.

Medicare Advantage HMO and PPO

Original claims: The maximum period for submission of Medicare claims is reduced to not more than 12 months. Claims received more than one calendar year from the date of service will be denied as being past the timely filing deadline.

Adjustment requests: The request must be received within 12 months from the date of service. Kaiser Permanente, as a Medicare carrier, may initiate an adjustment at any time.

Medicare and Medicaid overpayments must be reported and returned within 60 days after they are identified by the provider.

Commercial/PPO/POS
Original claims: The claim must be received by Kaiser Permanente within 12 months from the date of service.

Adjustment requests: The request must be received within 24 months from the date the claim was processed. Kaiser Permanente may initiate an adjustment within 24 months from the date the claim was processed (RCW 48.43.605).

Self-funded
Original claims: The claim must be received by Kaiser Permanente within 12 months from the original primary payment.

Adjustment requests: The request must be received within 24 months from the date the claim was processed (RCW 48.43.605). Kaiser Permanente may initiate an adjustment at any time.

Claims with COB involvement
Original claims: The claim must be received by Kaiser Permanente within 12 months from the original primary payment. If Kaiser Permanente receives information of primary insurance for a member, we may initiate an adjustment within 12 months from the date of notification.

Adjustment requests: The request must be received within 30 months from the date the claim was processed (RCW 48.43.605).

Self-funded: Timely filing limits vary. Please check with the employer group or the Provider Assistance Unit at 1-888-767-4670 for filing deadlines.

Health Plan Services Payor ID: 94320

6.2.23. Work-Related Injuries

Most member agreements with Kaiser Permanente exclude payment for any services associated with a job-related injury or illness. Promptly notify us whenever one of our members is injured in an on-the-job accident. Please use box 10A marked yes on a CMS 1500 form or occurrence code 04 listed in FL31 of a CMS 1450 form.

Kaiser Permanente will not be responsible for payment if the member's coverage agreement excludes work-related injuries from health plan coverage.

Agreements providing work-related injury coverage
There are some Kaiser Permanente member agreements that will cover on-the-job injuries if no L&I is available (please call the Provider Assistance Unit to check member's contract). In such cases, it is recommended the member stay within our network system in the event that the Washington State Department of Labor & Industry (L&I) or other self-insured workers' compensation carrier denies/rejects the claim.

If L&I denies the claim, send a copy of the denial along with the appropriate completed CMS form for payment to:

Kaiser Permanente Claims Administration
P.O. Box 30766
Salt Lake City, UT 84130-0766

Kaiser Permanente will not be responsible for payment if the workers' compensation carrier denies payment of services because required preauthorization was not obtained.

When you are providing services to members who are covered by Washington State Industrial Insurance:

- You must submit all necessary paperwork for the initial and subsequent visits to L&I or, for self-insured employers, to the appropriate carrier.
- Filing an accident report or rendering treatment to an injured worker constitutes acceptance of L&I rules and fees.

If the member works for an employer who is self-insured for work-related injuries or if the member cannot identify the workers' compensation carrier, you may call the L&I provider hotline.

Please refer to the Washington state Attending Doctor's Handbook available from L&I.

Contact the Other Party Liability Department or send the following information to assist with related claims to Kaiser Permanente Claims Administration:

- Member name and identification number
- Date of injury and claim number
- Name and telephone number of employer
- Nature of injury

If you are providing services to Kaiser Permanente members who are covered by Idaho Workers' Compensation:

1. The member must report the injury or illness to the employer. The employer then files a claim.

2. Contact the Idaho Industrial Commission Compliance Division to obtain the name of the insurance carrier that handles claims for the member's employer.

6.3. Provider Reconsiderations Conducted Within Kaiser Permanente

6.3.1. Reconsideration Time Frames for Kaiser Permanente Conducted Reconsiderations

Commercial members:

- **Member liability** – 180 days from the notification date of denial will follow the member appeals process.
- **Provider liability** – 24 months from the notification date of denial.
- **Coordination of benefits** – 30 months from the notification date of denial.

Medicare members:

- **Contracted providers** - 24 months from the notification date of denial.
- **Non-contracted providers** – 60 days from the notification date of denial will follow the member appeals process.
- **Coordination of benefits** – 30 months from the notification date of denial.

6.3.2. Medicare Advantage Non-Contracted Provider Appeal Rights

If you do not agree with the claim determination, you have the right to file an appeal. Written requests for appeal (reconsideration) of a zero-payment determination must be submitted within 60 calendar days of the date of this notice and must include a signed Waiver of Liability. The downloadable form is located within Section 50.1.1 of the CMS Parts C & D Enrollee Grievances, Organization/Coverage Determinations, and Appeals Guidance.

Reconsideration requests for denied claims, along with your supporting documentation (e.g. copy of this notice, clinical records) must be submitted in writing to:

Kaiser Foundation Health Plan of WA
Attn: Provider Appeals
PO Box 34593
Seattle, WA 98124-1593

6.3.3. Claims (Non-Authorization Related)

EXAMPLES

- Code review
- Pricing dispute
- Diagnosis code not commonly associated with the procedure
- Units billed do not match
- Modifier listed is invalid
- Timely filing

Note: Does not include all; purpose is for example only.

Claims denied with Member Liability follow the member appeals process.

If it's determined the claim needs to be re-processed, it will be adjusted to show up in a future remittance advice, and a written health plan decision letter will be sent. If the denial stands, you will receive a written health plan decision letter.

Note: If you are changing any data on your claim, submit your request as a corrected claim in order to promptly facilitate a payment determination.

WHERE TO SEND YOUR RECONSIDERATION

First Level Reconsideration:
First level reconsiderations may be called in to the Provider Assistance Unit, submitted electronically, or mailed to us.

- Phone: Call Provider Assistance Unit for review at 1-888-767-4670
- Electronic submission: Electronic Reconsideration Form
- Mail: Paper Reconsideration Form

Second Level Reconsideration:
If you do not agree with the decision regarding your first level reconsideration, you may submit a second level reconsideration to us electronically or by mail.

- Electronic submission: Request for Post Service Non-Authorization Reconsideration
- Mail: Paper Reconsideration Form

We do not have a way to process, download, save or store CDs. When hard copies or CDs are sent to the post office box, our Provider Reconsideration team does not have a way to monitor or ensure those documents are received. Please submit these requests via the Request for Post Service Non-Authorization Reconsideration form.

6.3.4. Claims (Authorization/Clinical Review Related)

EXAMPLES
- No authorization obtained
- Quantity exceeds authorization
- Date of service does not fall within dates of authorization
- Service exceeds authorization
- Type of Service/Place of Service does not match
- Service not medical necessary

Note: Does not include all; purpose is for example only.

HIPAA DENIAL CODE/REASON
- N435, N4351, and MA30
- N372

Note: Does not include all; purpose is for example only.

WHERE TO SEND YOUR RECONSIDERATION
Reconsideration:
- Electronic submission: Provider Reconsideration Request - Referrals and Medical Necessity Form (Online form and PDF available)
- Mail: Paper Reconsideration Form

All requests must include a detailed reconsideration letter stating the extenuating circumstances that prevented your facility from obtaining a provider authorization. Missing or incomplete information will result in rejection of your reconsideration request.

Inpatient/observation documents required for review:
- Registration and verification of insurance (if we are not notified of the stay)
- Procedures or operative reports
- ER notes
- Daily MD process notes
- History & physical
- Discharge summary
- Consultations

Providers seeking additional consideration will need to reach out to their organization's Contracting department to escalate. The Provider Assistance Unit and Provider Services are unable to assist if all levels of reconsideration have been exhausted.

6.3.5. Pre-Payment

Kaiser Permanente reviews commercial claims submissions $20,000 and greater prior to payment. If you disagree with the outcome of our pre-payment review, you may submit a reconsideration request.

EXAMPLES
- Inpatient facility charges
- Outpatient facility charges
- DRG contains outlier
- Percentage of charge payment reimbursement

Pre-payment review policies:
Prepayment Bill Review - Line Item Deduction (LID)
Prepayment Bill Review - Medical Necessity

When submitting the requested itemization/medical records or reconsideration (PDF format preferred for optimized file size), please include the following:
- Member's name
- Patient's 8-digit member ID
- Date of service
- Claim number

MEDICAL RECORDS AND ITEMIZATION SUBMISSIONS REASON CODE
- N366, N26, and N221

Note: Does not include all; purpose is for example only.

MEDICAL RECORDS AND ITEMIZATION SUBMISSIONS
If your claim is denied for itemization and/or medical records due to the pre-payment review, submit records for review (PDF format preferred for optimized file size):
- Electronic submission: Submission of claim supporting document
- Email: pre-pay-inbox@kp.org

DENIAL CODE/REASON
- N661, N163, N22, and M15

Note: Does not include all; purpose is for example only.

WHERE TO SEND YOUR RECONSIDERATION
First Level Reconsideration:

- Electronic submission: Request For Post Service Non-Authorization Reconsideration
- Email: pre-pay-inbox@kp.org
- Mail: Pre-Payment Reconsideration Form

Second Level Reconsideration:
If your request does not include additional or new information, your request will not be reviewed.

- Electronic submission: Request For Post Service Non-Authorization Reconsideration
- Email: pre-pay-inbox@kp.org
- Mail: Pre-Payment Reconsideration Form

For all claims pre-payment questions and inquiries

Please email the Claims department directly at pre-pay-inbox@kp.org for questions or assistance with claims. For the quickest response, do not submit questions to your Provider Services Consultant, as they will simply forward questions to this inbox on your behalf, thus slowing your wait time for an answer.

If both levels of reconsideration have been reviewed, and denials upheld, providers have exhausted their allowance of reconsideration attempts and the reconsideration process is no longer available for consideration.

Providers seeking additional consideration will need to reach out to their organization's Contracting department to escalate. The Provider Assistance Unit and Provider Services are unable to assist if all levels of reconsideration have been exhausted.

6.4. Reconsiderations Conducted Outside Kaiser Permanente

6.4.1. Diagnosis Related Group (DRG) Payment and Review (Cotiviti)

We may perform DRG reviews on claims which are reimbursed by MS-DRG, APDRG, and APR-DRG to validate that the diagnosis and procedural information leading to the DRG assignment is supported by the medical record. The purpose of DRG validation is to ensure diagnostic and procedural information and discharge status of the patient, as coded and reported by the hospital on its claim, matches both the attending physician's description

and the information contained in the patient's medical records. Please refer to the DRG payment and review policy located under More Resources on this page for more information on these claims reviews.

We have engaged the services of Cotiviti to conduct these inpatient claims reviews on our behalf. If you have received a communication from Cotiviti and Kaiser Permanente requesting claims records, please refer to the Cotiviti Upload Portal Job Aid for instructions on how to submit the requested records via Cotiviti's upload portal.

EXAMPLES
- DRG (Diagnosis Related Group) Reviews

DENIAL CODE/REASON
- N221 and N10

Note: Does not include all; purpose is for example only.

WHERE TO SEND YOUR RECONSIDERATION
- Mail: Cotiviti
  C/O Cotiviti-6170 731
  Arbor Way Box 12017
  Blue Bell, PA 19422
- FAX: 801-683-1762
- Secure Portal at www.submitrecords.com
- Upload documents to: Cotiviti Provider Upload Portal
- For status or questions contact Cotiviti at: 770-379-2166

Note: All inquiries regarding Cotiviti claims reviews must go directly to Cotiviti

6.4.2. Optum

Kaiser Permanente has engaged Optum to verify the extent and nature of services rendered for the patient's condition and that claims are coded correctly for the services provided. If you receive a letter requesting records, please follow the instructions provided in the letter.

If records are not received within 30 days (commercial) or 54 days (Medicare), the claim will be denied with HIPAA remark code M127. If the medical records are reviewed and the lines denied, the claim will deny with HIPAA remark code N163. A letter will be sent to you with the reasons for denial.

If you disagree with the decision and wish to file an appeal, the most efficient way to get the records reviewed quickly is to use the case number that is sent with the letter containing the denial rationale. Requests for reconsideration must be submitted directly to Optum.

Please see the Optum Integrity Portal job aid for instructions on how to upload documents electronically in response to a request for documentation.

Note: All inquiries regarding Optum claims reviews must go directly to Optum

Optum Customer Service: 1-877-687-2062

6.4.3. WA Balance Billing Protection Act

The WA Balance Billing Protection Act requires Kaiser Permanente to review claims to ensure that fees charged by non-participating providers are appropriate for emergent services.

EXAMPLES

- The charge(s) for this service was processed in accordance with WA State Balance Billing regulations. As such, any amount identified with OA, CO, or PI cannot be collected from the member and may be considered provider liability.

DENIAL CODE/REASON

- N830

WHERE TO SEND YOUR NEGOTIATION

First Level:

- Reconsideration time frame: Providers have 30 days from the notification date of denial
- Mail to:
  Kaiser Permanente
  Attn: Balance Billing Arbitration
  P.O. Box 30766
  Salt Lake City, UT 84130

Second Level Arbitration:

- If you do not agree with the outcome of the negotiation, you have the right to request arbitration of the claim

- Reconsideration Time Frame: Providers have 40 days from the notification date of denial
- All inquiries regarding Second Level Reconsiderations for the Balance Billing Protection Act will need to go directly to the OIC
    - Office of Insurance Commissioner at: 1-800-562-6900
    - Email to: Balance-Billing-Arbitration@kp.org

6.4.4. Federal No Surprise Billing Act

These claims are reviewed by Multiplan to ensure that the Allowed Amount is the qualifying payment for each item or service. The qualifying payment amount applies for purposes of the recognized amount. The qualifying payment amount was determined in compliance with applicable federal or state regulation.

EXAMPLES

- The Federal No Surprise Billing Act qualified payment amount (QPA) was used to calculate the member's cost share(s).

DENIAL CODE/REASON

- N860

WHERE TO SEND YOUR NEGOTIATION

- Negotiation time frame: If you wish to initiate a 30 business-day open negotiation period for purposes of determining the amount of total payment, as applicable, please contact the appropriate person or office.

If the negotiation period does not result in an agreement as to the amount of total payment, you may initiate the independent dispute resolution process within 4 business days after the end of the open negotiation period.

Claims with note(s) - PAID PER MULTIPLAN

- Email to: NSAIDR@multiplan.com
- Multiplan at: 1-888-593-7427

Claims with note(s) - PAID PER QPA (FH) or PAID PER QPA (A)

- Email to: KPWA-NSA-ONP@KP.org
- Paper Negotiation Form

AIR AMBULANCE CLAIMS

Multiplan reviews Air Ambulance claims to ensure the qualifying payment amount was determined to comply with applicable federal requirements.

EXAMPLES

- Pricing
- Allowed Amount

WHERE TO SEND YOUR NEGOTIATION

Negotiation time frame: 30-business-day open negotiation period.

- Email to: Air-Ambulance-Appeals@kp.org
- Or by calling Relation Insurance Services (formerly Employer's Mutual Incorporated) at 888-505-0468.

7. PATIENT CARE

7.1. Clinical Guidelines

See our Clinical Guidelines for helpful resources on patient care.

7.2. Mental Health

Kaiser Permanente contracts with mental health providers who are committed to delivering high quality, evidence-based care. This includes using measurement-based care to track symptom improvement over time and adjust treatment when patients are not progressing, as well as graduating patients from treatment when they have achieved symptom reduction and met their treatment plan goals. We value providers who focus on meaningful outcomes and support patients in successfully completing their episode of care and sustaining their wellbeing. We seek contracted providers who align with this philosophy of care so that all members have equitable access to high quality, affordable care.

7.2.1. Mind Phone

Kaiser Permanente offers the Mind Phone telephonic psychiatric consultation phone line to all medical and mental health practitioners in the Kaiser Permanente delivery system. Mind Phone is for provider-to-provider consultation only and is not for patient use.

Practitioners can call the Provider Assistance Unit at 1-888-767-4670 to be connected to Mind Phone on weekdays from 8 a.m. to 5 p.m. Messages will be responded to within 30 minutes. Providers can also send consultation requests via Epic staff messaging to Mind Phone. Holiday, after-hours, and weekend consultation is not available.

Mind Phone psychiatrists are available to assist with:

- Reaching a diagnosis
- Making psychotropic medication recommendations and decisions
- Providing information about psychotropic medications
- Deciding which psychotropic medications (including non-formulary medications) to continue for a patient transferring their care to Kaiser Permanente
- Deciding on next steps in treatment

- Deciding whether to make a referral
- Addressing questions about our mental health and wellness service line and our delivery system
- Responding to a patient or family question
- Coordinating care (deciding to whom to refer, who handles which aspects of care, etc.)
- Deciding which psychotropic medications (including non-formulary medications) to continue for a patient transferring their care to Kaiser Permanente

For referrals, members must call the Mental Health Access Center (MHAC) at 1-888-287-2680. Providers can also enter a referral order in Epic – Ref Mental Health – and MHAC will attempt to contact the patient to process the referral request.

MHAC arranges and authorizes payment for mental health and addiction and recovery services that are determined to be:

- Medically necessary, and
- Covered by the member's medical coverage agreement

Patients do not need a referral from their primary care physician to obtain services. Some services require prior authorization for coverage.
Patients are encouraged to call Member Services to review their benefits and obtain member cost share. To access mental health and addiction and recovery services, members should contact the Mental Health Access Center prior to seeking services.

7.2.2. Measuring Quality of Mental Health Care

We are committed to the highest standards of clinical quality. As an organization, we measure our quality in several ways. We are accredited by the National Committee for Quality Assurance (NCQA) and have adopted the following quality measures using the Healthcare Effectiveness Data and Information Set© (HEDIS) and various instruments for measurement:

- How quickly members receive initial service from time of authorization – This is captured through claims data. NCQA and Kaiser Permanente require patients to be seen within 10 business days for routine care, 48 hours for urgent care, and immediately for emergent care.
- How quickly a member is seen by their treating provider following an inpatient psychiatric hospitalization – This is captured through claims data. We require a visit with a mental health professional within 7 days of discharge. HEDIS also measures patients seen within 30 days of discharge. If you have an active patient in psychiatric hospitalization,

please ensure that you meet with your patient within 7 days of discharge or ensure a visit with another mental health provider. The Mental Health Access Center (MHAC) will provide support to help meet this very important quality indicator by assisting in the coordination of post-hospitalization visits through our hospital liaison team as needed.

- How satisfied patients are with their services through Kaiser Permanente and their treating provider – This is captured through satisfaction surveys to patients of high-volume network mental health providers using the Patient Experience Survey form.
- How well your records meet our standards – This is captured by random records review of identified providers using the Provider Treatment Record Review form.
- How well your offices meet environment standards – Office appearance and safety are of high value to our members. Therefore, providers are expected to meet Kaiser Permanente minimum standards. If there have been three or more complaints, Kaiser Permanente will assess the office using the Office Environment Assessment form.

7.3. Complementary and Alternative Medicine

Kaiser Permanente provides options for members to access complementary and alternative care and manipulative therapy services, otherwise known as complementary and alternative medicine (CAM). These services include acupuncture, naturopathy, massage therapy, and manipulative therapy services provided by chiropractors. Eligibility varies based on the member's health plan and benefit level.

Most plans cover a limited number of self-referred acupuncture, naturopathy, and manipulative therapy visits. Most plans also cover massage therapy visits with a referral from the primary care provider or personal physician. Providers and members can confirm coverage and visit limits by contacting Member Services at 1-888-901-4636. Providers are encouraged to verify coverage before delivering services.

Kaiser Permanente contracts with WholeHealth Networks, Inc. (WHN), a subsidiary of Tivity Health Support, LLC (Tivity), to manage our network of chiropractors, naturopaths, acupuncturists, and massage therapists. All contracted chiropractic and CAM providers meet Kaiser Permanente's credentialing standards and state or local licensing requirements. Members may identify these providers by using our Provider Directory.

Contracted CAM providers should refer to the Tivity Kaiser Permanente Provider Manual Supplement located on the Tivity website for further information.

7.4. Medical Transportation

Kaiser Permanente stresses using appropriate and prudent professional judgment in your decisions regarding medical transportation.

7.4.1. Emergency Situations

In situations requiring immediate ambulance transfer, call 911 if you believe the situation is emergent or life-threatening and requires transfer within the next 10 minutes.

7.4.2. Non-Emergency Situations

In situations requiring ambulance transfer within 30 to 60 minutes or more, refer to the EMS Call Guide for a list of providers in your area.

7.4.3. Medical Transportation Management (formerly Access2Care)

Some of our Medicare Advantage plans include a transportation services benefit that provides transportation to/from a network medical or dental facility. This benefit is limited to a specific number of trips per calendar year, and the ride must be with our designated network provider. Please advise members to confirm their benefits prior to use. The EMS Call Guide includes Medical Transportation Management (formerly Access2Care) contact information for your members.

7.5. Care Management Programs

Kaiser Permanente's Complex Care Management (CCM), Next Step Care (NSC), and High-Risk Pediatric Care Management programs support members with complex, high-risk medical and social needs. These members often require intensive coordination due to multiple or poorly controlled chronic conditions.

The programs meet NCQA standards and are essential to health plan accreditation. Members are identified through provider referrals, hospital discharge planning, internal programs, and predictive risk registries.

7.5.1. Key Services

- Enrollment of high-risk members
- Comprehensive risk assessments
- Individualized care planning
- Barrier identification and resolution
- Care coordination and navigation
- Benefit utilization support
- Community resource referrals
- Education for members and caregivers
- Transitional care support after hospital, SNF, or ED discharges
- Care gap closure

7.5.2. Program Outcomes

- Fewer preventable hospital and ED visits
- Improved member and provider experience
- Increased care gap closure
- Improved self-management of chronic conditions

7.5.3. How to Refer

- Submit a referral to REF CARE MANAGEMENT or
- Call 1-866-656-4183

7.6. Home Health and In-Home Palliative Services

In the Puget Sound region (excluding Olympia), the Kaiser Permanente Continuing Care Division provides home health services. In Olympia, Northwest Washington, and East of the Cascades, these services are available from contracted agencies.

7.6.1. Scope Of Services

Home health includes intermittent interdisciplinary home care services to members twenty-four (24) hours a day, seven (7) days a week. Home health covered services include:

- Skilled nursing services
- Physical, occupational and speech therapy services
- Medical social work services
- Home health aide services

Medicare guidelines are used to determine if a member meets the requirements to receive skilled home health care services.

Members accepted for admission to Home Health will receive certain core services if the patient is a Kaiser Permanente Health Plan Member or under the care of a Kaiser Permanente physician. Patients will be accepted for care without discrimination on the basis of race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, or place of national origin. Members will be accepted for care based on the adequacy and suitability of organization personnel, resources to provide required services, and the reasonable expectation that the patient's medical, nursing, rehabilitative, and social needs can be adequately met in the patient's place of residence.

Specialty services not provided by the home health agency include but are not limited to: pediatric care, specialized lymphedema treatment, invasive procedures outside of the home health scope, chemotherapy or radiation therapy, blood transfusions or dialysis, long-term custodial care, 24-hour continuous skilled nursing care, transportation services, and services requiring specialized equipment not portable or feasible in the home setting.

7.6.2. Admission Criteria

Kaiser Permanente's criteria for admission to home health services are based on the federal regulations for the Medicare home health benefit and MCG guidelines.

7.6.3. Arranging for Services

Submit a Home Health referral via EPIC Order Entry, fax or send a request for authorization using the Affiliate Link referral request tool.

Home Health and Hospice Referral Intake Office
206-326-4444
Toll-Free: 1-800-332-5735
Fax: 1-877-516-5848

7.6.4. Contracted Providers

Contracted providers are expected to follow the standards of care outlined in Home Care Services guideline.

7.7. Hospice Services

In the Puget Sound region (excluding Olympia), the Kaiser Permanente Continuing Care Division provides hospice services. In Olympia, Northwest Washington, and East of the Cascades, these services are available from contracted agencies. The purpose is to make available short-term inpatient care for pain control and acute and chronic symptom management, continuous care in the home when needed for acute symptom management crises, and respite care via the hospice provider.

7.7.1. Scope of Services

Hospice Services includes services made available to members that is available twenty-four (24) hours a day, seven (7) days a week. Hospice covered services include:

- Short-term inpatient care for pain control
- Acute and chronic symptom management
- Continuous care in the home when needed for acute symptom management crises
- Respite care via hospice providers

7.7.2. Admission Criteria

Kaiser Permanente members qualify for hospice care using the same criteria as the Medicare hospice benefit. Contracted hospice agencies must bill Medicare directly for Kaiser Permanente Medicare members receiving hospice care.

7.7.3. Arranging for Services

Please submit a Home Health or Hospice referral via EPIC Order Entry, fax or send a request for authorization using the Affiliate Link referral request tool.

Home Health and Hospice Referral Intake Office
206-326-4444
Toll-Free: 1-800-332-5735
Fax: 1-877-516-5848

7.7.4. Contracted Providers

Contracted providers are expected to follow the standards of care outlined in the Hospice Care Service guideline.

7.7.5. Hospice Information for Medicare Part D

The Hospice Information for Medicare Part D form is used by providers for documenting Part D coverage of drugs for members who are Medicare beneficiaries enrolled in hospice. This form facilitates coordination between Part D plans, hospices, and pharmacists. Two primary uses are to document that a drug is unrelated to a Medicare beneficiary member's terminal prognosis, and to convey the Medicare beneficiary member's change in hospice status.

When providers submit this form prospectively, it ensures that Kaiser Foundation Health Plan of Washington will be able to determine whether a drug is related or unrelated to the Medicare beneficiary member's terminal prognosis and, therefore, whether the drug is covered under Part D. The form may also be used for contracted providers to communicate and update the medications list from the Medicare beneficiary member's plan of care.

- Hospice Information for Medicare Part D Form
- Instructions for Providers Completing the Form
- NCPDP Instructions for Hospice Status

Submit the form to:
Kaiser Foundation Health Plan of Washington
Pharmacy Administration Help Desk
Mailstop: GSE-B2s-01
2921 Naches Ave S.W. Renton, WA 98057
Fax: 1-866-510-1765

7.8. Nursing Home Care – Long Term Care (Custodial Care)

Kaiser Permanente does not cover residential-level or custodial-level nursing home care. Depending on the member benefit, Kaiser Permanente does cover intermittent therapy needs identified in the LTC/Custodial Care setting. The therapist in the nursing home will submit orders and therapy evaluations to Nursing Home Services to get authorization for this service.

Once a member selects a nursing home for long term care, their primary care provider or personal physician will work with clinic social worker to request placement. Below is a list of most frequently required information. Please contact the nursing home directly to determine all required paperwork.

- Physician admission orders
- Specific diagnoses for all medications
- Current history and physical or emergency department evaluation
- Therapy evaluations and recent progress notes, if applicable

The personal physician or their representative must notify Nursing Home Services of custodial placement. Leave a voice message at 1-800-887-3873 with the following information:

- Member name
- Kaiser Permanente member ID
- Date of birth
- Name of nursing home
- Admission date

7.9. Skilled Nursing Facility Care

Kaiser Permanente Nursing Home Services coordinates skilled nursing facility (SNF) services.

Kaiser Permanente members may receive inpatient care in one of our in-network SNFs when that care is preauthorized by a Kaiser Permanente Care Management representative. We must review all SNF admissions to determine whether the member meets SNF criteria to issue an authorization or a notice of non-coverage. To meet Medicare Criteria, patients must have a daily nursing or therapy need.

7.9.1. Arranging for Services

All SNF placements require prior authorization. If the member is in the hospital, discharge planners will arrange for SNF placement. If a member is in your office, request prior authorization online or contact Nursing Home Services toll-free at 1-800-887-3873.

To determine whether the member meets clinical criteria, has a SNF benefit, and which facility can meet their care needs, we will need the following information:

- Member name and Kaiser Permanente ID number
- Reason for referral
- Physician admission orders
- Specific diagnoses for all medications
- Current history and physical or emergency department evaluation
- Therapy evaluations and recent progress notes, if applicable
- Pre-admission screening resident review (PASRR)

7.9.2. Contracted Provider Expectations

Contracted providers are expected to follow the standards of care outlined in the Skilled Nursing Facility Services guideline.

Participation in Quality Reporting for Nursing Home Care

In addition to the commitment to Quality Improvement noted in the Provider Responsibilities section of this manual, contracted providers are expected to participate in Quality Improvement activities and provide Quality Reporting on a quarterly basis, utilizing quality metrics, targets and national benchmarking data, when available (“Quality Standards”).

The Provider Quality Scorecard will be submitted to Kaiser Permanente, including its most recent CMS Nursing Home Compare overall rating score,

as published and updated from time to time by CMS, no later than the 15th of month following each calendar quarter.

The Provider Quality Scorecard will include metrics, Quality Standards and targets that include but are not limited to those outlined below. No more than annually will the targets be reset in accordance with the contracted providers' performance.

To assist you in meeting the performance goals, you may request quarterly information from Kaiser Permanente, which will provide the status of its current performance. In the event you experience a substantial increase in the acuity or complexity of Members referred by Kaiser Permanente, which may adversely impact your ability to meet the Targets and/or Quality Standards, you will notify Kaiser Permanenete, as soon as practicable.

Provider Quality Scorecard

Kaiser Permanente Provider Quality Scorecard								
Current Year	*202X*							
Metrics	Points earned per Metric		-3	0	3	5	Year Average	Points per Metric
Quality and Staffing		***Quality Standards****				***Target Performance Goal***		
CMS Star Rating								
Direct Nursing PPD								
Therapy Hours per Week								
Clinical Metrics per 1000						***Target Performance Goal***		
Facility Acquired Infections								
UTI w/catherter Rate								
UTI w/o Catheter Rate								
Falls with Injury								
Facility Acquired or Worsening Pressure Ulcers								
Mediation Errors								
Triple AiM (Q, Set, Afford)						***Target Performance Goal***		
Returen to Acute Rate (SNF-->Hosp-->SNF)								
30-Day readmission post-SNF								
Quality Standards are based on national benchmarks, when available, and may be updated from time-to time.								

7.10. Patient Education and Support Services

Kaiser Permanente has many population-based programs and services for members with certain health conditions, such as diabetes, heart health, and depression. Kaiser Permanente continually evaluates the need to develop additional population-based care programs.

Kaiser Permanente offers several types of educational and support services for our members, including:

- Establishing a health profile to support member goals
- Case management for complex care needs

- Health and wellness library covering over 4,000 medical conditions, symptoms, and medical procedures
- Tobacco cessation programs for members 13 and older
- Wellness coaching via phone
- Classes, support groups and fitness events at our medical centers and in our community
- Patient education brochures, handouts, booklets, and self-management resources, printable in easy-to-read PDF format
- 24/7 nursing advice hotline
- Community resources connection assistance

By registering for an account on www.kp.org, members may access all the resources listed above at any time. Additionally, many of these resources are included at no cost in member plans. Members may verify coverage by calling Member Services at 1-888-901-4636.

Providers who are interested in connecting their Kaiser Permanente patients with these resources can call the Provider Assistance Unit at 1-888-767-4670 for further information.

7.11. Specialty Services

Kaiser Permanente offers many specialty services for its members. Please see the Specialty Services page on our provider site for more information on how we can support your patients.

7.12. Women's Health Care Services

7.12.1. Coverage

Women's health care services including maternity care, covered reproductive health services, preventive services (well care) and general examinations, gynecology care, and follow-up visits (check the member's Evidence of Coverage to understand limitations in this area) may be obtained from Kaiser Permanente or Kaiser Permanente network providers without prior authorization. Providers include general and family practitioner, physician's assistant, gynecologist, certified nurse midwife, licensed midwife, doctor of osteopathy, pediatrician, obstetrician or advance registered nurse practitioner.

7.12.2. Home birth

A member's health plan may cover part or all home-birth services. A Kaiser Permanente member may self-refer to a contracted home birth practitioner for an initial examination to determine whether she meets our criteria as a candidate for home birth. We do not require prior authorization for ancillary services associated with typical, low-risk prenatal care.

7.12.3. Infertility/Fertility Treatment

Services related to conception by artificial insemination may be covered on commercial plans. Other infertility/fertility treatments are generally exclusions on most plans. Large groups (fully insured and self-funded) may purchase a benefit to provide coverage of additional treatments for infertility/fertility beyond artificial insemination. Members should verify coverage to confirm their benefits.

7.12.4. Pregnancy Termination

Kaiser Permanente will provide payment for abortion services and related ancillary services on most plans. Members should verify coverage to confirm their benefit.

The Federal Employees Health Benefits Program (FEHBP) and the Postal Service Employees Health Benefits Program (PSHB) do not offer coverage of abortion procedures except in the following limited circumstances according to the approved plan brochure:

- The incidence of the pregnancy being a result of an act of rape or incest.
- The case where a woman suffers from a physical disorder, physical injury or physical illness, including a life-endangering physical condition caused by or arising from the pregnancy itself, that would, as certified by a physician, place the woman in danger of death unless an abortion is performed.

8. PHARMACY SERVICES

8.1. Drug Formulary

The Kaiser Permanente drug formulary is a list of medications that are usually covered under a member's pharmacy benefit. The Pharmacy & Therapeutics Committee develops the formulary, the cornerstone of medication therapy, quality assurance, and cost-containment efforts.

8.1.1. Drug Formulary Exception Requests

You may request coverage for medications requiring prior authorization and for drugs not included in the formulary. To submit a formulary exception on behalf of a patient, call the Drug Benefit Help Desk at 1-800-729-1174, option 1, or fax to 1-866-439-0050. Patients can also submit a formulary exception request via the online form on the secure member website.

8.1.2. Drug Formulary Recommendations

To make formulary recommendations, please contact your Network Medical Director or a Kaiser Permanente clinical pharmacist. Requests to add a drug to the formulary must include reasoning, suggestions for appropriate restrictions or usage guidelines, and references to relevant literature.

8.1.3. Formulary Tools

Both the Kaiser Permanente and Kaiser Permanente Medicare Part D Drug formularies are searchable. You can download the commercial formulary to your smart phone or access it online through Epocrates. You may also request a hard copy of the formulary from Member Services.

- Drug formulary search: Search for drugs by name or therapeutic class.
- Epocrates downloadable formulary: Check formulary status and prior authorization criteria for medications covered under the pharmacy benefit.
- Latest Medicare Part D formulary updates
- Kaiser Permanente commercial quantity limit drug list
- Limited distribution medications

8.2. Pharmacy & Therapeutics Committee

Kaiser Permanente's Pharmacy & Therapeutics Committee is a statewide group of contracted clinicians who evaluate medications for the organization's formulary by relying on long-term patient interests, evidence-based review, and cost-benefit considerations; they prioritize clinical data over expert opinion and regularly assess both new and existing drugs across all therapeutic areas. Decisions about which medications to review are influenced by provider demand, frequency of requests, and the availability of strong published research or controlled trials. Through this process, the committee selects the safest, most effective, and most cost-efficient agents in each drug class. You can review the decisions made at the most recent committee meeting in the quarterly Formulary Decision Highlights.

8.3. Pharmacy Drug Benefit Help Desk

The Pharmacy Drug Benefit Help Desk reviews and determines coverage for formulary medications requiring prior authorization and non-formulary drug requests for Kaiser Permanente Washington members (Commercial and Medicare) based on plan benefits and established medical necessity criteria.

8.3.1. How to Submit a Prior Authorization Request

Providers may submit PA requests using one of the following methods:

- Electronic Prior Authorization (Preferred) – Submit directly through your Electronic Health Record (EHR) or ePA platforms such as CoverMyMeds.
  • Submit completed clinical information electronically.
  • Determinations are returned electronically to your EHR (fax notification also provided).
- Fax – Fax completed requests with supporting clinical documentation to 1-866-439-0050.
- Phone – Call 1-800-729-1174 to provide required clinical rationale.
- Chronic High-Dose Opioid Therapy Prior Authorization Form: Commercial members on chronic opioid therapy >=90 mg MED/day require prior authorization to ensure their care plan supports safe use of opioids.

8.3.2. Clinical Documentation Requirements

To avoid delays or denials, include:

- Clear documentation of medical necessity
- Relevant treatment history and failure reasons
- Supporting labs or chart notes when applicable

Incomplete information may result in denial.

8.3.3. If a Prior Authorization Request is Denied

Denial letters include the reason for denial, applicable criteria, preferred alternatives (if available), and appeal rights. Before submitting an appeal:

- Review covered alternatives listed in the denial letter.
- Submit additional or clarifying clinical information for reconsideration, if available (fax or call).

Pharmacy Drug Benefit Help Desk
Phone: 1-800-729-1174
Fax: 1-866-439-0050
Email: rxbene1@kp.org
Hours of Operation: Monday–Friday 8:00 a.m.–6:00 p.m.;
Saturday–Sunday 8:30 a.m.–5:00 p.m.

8.4. Prior Authorization Review Timeframes

For standard requests: If Kaiser Permanente contacts the provider for additional information, please respond within 5 calendar days. If the supporting statement is sufficient, we will render our decision and provide notification within 4 calendar days of receipt of your statement.

8.4.1. Medicare

- Expedited requests: No later than 24 hours from receipt of the prescriber's supporting statement.
- Standard request: No later than 72 hours from receipt of the prescriber's supporting statement.

8.4.2. Commercial

- Non ePA expedited requests: No later than 2 calendar days from receipt of request if all information is provided.
- Non ePA standard requests: No later than 5 calendar days from receipt of request if all information is provided.
- ePA expedited requests: No later than 1 calendar days from receipt of request if all information is provided.
- ePA standard requests: No later than 3 calendar days from receipt of request if all information is provided.

For Expedited requests:
If Kaiser Permanente contacts the provider for additional information for an expedited request, please respond within 2 calendar days. We will render our decision and provide notification within 2 calendar days of receipt of your statement.

8.5. Prior Authorization for Injectable Drugs

Kaiser Permanente requires prior authorization of certain injectable medications administered in the office or home infusion setting. These reviews ensure that benefits are adjudicated and that use is in line with Pharmacy & Therapeutics Committee criteria.

Current lists of injectable drugs requiring prior authorization:
- Non-Medicare Injectable Drugs Requiring Prior Authorization
- Medicare Part B Office-Administered Drugs Requiring Prior Authorization
- Medicare Part B Drugs Requiring Step Therapy

Kaiser Permanente's preferred biosimilar products:
- Biosimilars: Preferred Products and Management Strategies

Policies regarding site of care and site of service:
- Utilization Management Policy: Site of Care: Infusion Therapy and Clinic Administered Medicines
- Utilization Management Policy: Site of Service: Infusion Therapy and Clinic Administered Medicines

8.6. Self-Administered Medications

The Self-Administered Medications Policy includes the criteria used to identify that a medication is self-administered and provides a list of medications that are not covered under the medical benefit.

8.7. Home Infusion Pharmacy Services

Kaiser Permanente contracts with infusion nursing services providers to deliver infusion therapy services to our members twenty-four (24) hours a day, seven (7) days a week. These providers initiate infusion services within twenty-four (24) hours of the order receipt.

Referrals: To refer a patient to Kaiser Permanente Home Infusion Pharmacy, follow the instructions in the referral job aid or fax a prescription referral to 206-326-2139.

- Home Infusion Pharmacy Formulary
- Home Infusion Referral Job Aid
- Home Infusion Referral Forms
- Utilization Management Policy - Site of Care: Infusion Therapy and Clinic Administered Medicines

Contact us:
Telephone: 206-326-2990 (stay on the line to speak to pharmacy staff)
Fax: 844-785-3104

Hours of operation:
Monday through Friday, 8:30 a.m. to 5:00 p.m.

8.8. Specialty Pharmacy Services

Kaiser Permanente Washington Specialty Pharmacy works closely with Kaiser Permanente Washington members and providers to manage high-cost medications that require close supervision and monitoring for serious chronic conditions. We verify benefits, assist with prior authorization, review patients' medications for potential interactions, assess possible side effects, monitor for treatment efficacy, and schedule regular follow-up calls with your patients to support adherence and treatment success. Should any problem arise, we will contact your office to notify you of the situation.

Referral forms are required for Hepatitis C and Hizentra (also see our Hizentra Sample Program Request Form). Please complete the applicable patient referral form and submit it via fax to us at 1-800-340-4230.

See our Specialty Pharmacy Services information page for more information about our scope of services.

Contact us:
Telephone: 800-483-3945
Fax: 800-340-4230

Hours of operation:
Monday through Friday, 8:30 a.m. to 5:00 p.m.